

MODULE 1: ROLE OF THE CERTIFIED NURSING ASSISTANT

The Nurse Aide Role, Scope of Practice, Professionalism & the Healthcare Team

GA Healthcare Training & Consulting

Certified Nursing Assistant (CNA) Program

Estimated Completion Time: 2–3 Hours

Textbook Reference: Nursing Assistant Care: The Basics, 6th Ed. (Hartman) | Ch 1, pp. 1–20; Ch 10, pp. 249–255

Federal & State Core Curriculum Alignment: Part I.F — Legal and Ethical Behavior and Scope of Practice; Part I.A — Chain of Command; Part I.D — Promoting Residents' Independence

MODULE TITLE

Role of the Certified Nursing Assistant

MODULE OVERVIEW

The certified nursing assistant is the member of the healthcare team who spends the most time with the resident. Because of that closeness, the CNA is often the first person to notice that something has changed — a new bruise, a lost appetite, a resident who is quieter than usual. What the CNA observes and reports directly affects the care the resident receives.

This module introduces the nurse aide role: what a CNA does, where the role sits within the healthcare team, what the law in Georgia permits a CNA to do, and what a CNA must never do. It also introduces the professional behavior, work ethic, and chain of command that every other module in this program builds on.

The CNA role includes:

- Assisting residents with activities of daily living
- Observing and reporting changes in condition
- Providing safe, respectful, person-centered care
- Supporting resident independence and dignity
- Documenting care accurately
- Working as part of the interdisciplinary healthcare team

This module teaches nurse aides how to:

- Describe the purpose and responsibilities of the nurse aide role
- Work within the Georgia CNA scope of practice
- Use the chain of command correctly

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- Demonstrate professionalism, dependability, and accountability
- Understand the requirements for certification and registry listing

LEARNING OBJECTIVES

By the end of this module, learners will be able to:

1. Define the role of the certified nursing assistant in long-term care.
2. Describe the responsibilities of the CNA during a typical shift.
3. Identify the members of the interdisciplinary healthcare team and the role of each.
4. Explain the Georgia CNA scope of practice and distinguish tasks that are within scope from tasks that are not.
5. Describe delegation and explain who may delegate a task to a nurse aide.
6. Apply the chain of command to report observations, concerns, and changes in resident condition.
7. Demonstrate professional behavior, appearance, dependability, and accountability.
8. Explain the CNA's role as an observer and reporter and why timely reporting matters.
9. Describe the certification requirements for nurse aides in Georgia and the purpose of the Nurse Aide Registry.
10. Identify career advancement pathways available to a certified nursing assistant.

LESSON 1: WHAT IS A CERTIFIED NURSING ASSISTANT?

A certified nursing assistant is an unlicensed member of the nursing team who has completed a state-approved training program, passed a competency evaluation, and been listed on the state nurse aide registry. The CNA provides hands-on, direct care to residents under the supervision of a licensed nurse.

CNAs work in a variety of settings:

- Nursing homes and long-term care facilities
- Hospitals and rehabilitation centers
- Assisted living communities
- Home health agencies
- Hospice programs
- Adult day health programs

Titles used for this role vary by setting — nurse aide, nursing assistant, patient care technician, geriatric aide, home health aide — but the core responsibility is the same: safe, dignified, direct resident care under nurse supervision.

Why the CNA role matters

The CNA spends more time at the bedside than any other team member. This makes the nurse aide the eyes and ears of the healthcare team. A change noticed and reported early can prevent a

fall, a hospitalization, or a pressure injury. A change missed or not reported can cause serious harm.

LESSON 2: RESPONSIBILITIES OF THE NURSE AIDE

Personal care and activities of daily living (ADLs)

- Bathing, showering, and perineal care
- Grooming, oral care, shaving, hair and nail care
- Dressing and undressing
- Toileting and incontinence care
- Assistance with eating, drinking, and hydration
- Transfers, positioning, turning, and ambulation

Measurement and observation

- Vital signs — temperature, pulse, respirations, blood pressure
- Height and weight
- Intake and output
- Skin condition and early signs of breakdown
- Changes in appetite, mood, behavior, mobility, or level of function

Environment and safety

- Keeping the resident's room clean, uncluttered, and safe
- Keeping the call light within reach at all times
- Following infection control and standard precautions
- Reporting hazards, faulty equipment, and unsafe conditions

Communication and documentation

- Reporting observations to the supervising nurse promptly and accurately
- Documenting care provided in the record, according to facility policy
- Participating in shift report and handoff communication
- Communicating respectfully with residents, families, and staff

Emotional and psychosocial support

- Listening to residents and treating each one as an individual
- Supporting resident choice, privacy, and independence
- Encouraging participation in activities and social contact
- Respecting cultural, spiritual, and personal preferences

LESSON 3: THE HEALTHCARE TEAM

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The CNA is part of an interdisciplinary team. Each member contributes a different expertise, and the resident — not the staff — is at the center of the team.

- Resident and family — the center of the care plan; participate in decisions about care
- Registered Nurse (RN) — assesses residents, develops the care plan, delegates and supervises
- Licensed Practical Nurse (LPN/LVN) — provides nursing care and supervises nurse aides under RN direction
- Certified Nursing Assistant — provides direct care; observes and reports
- Physician / Nurse Practitioner — diagnoses and orders treatment
- Physical, Occupational, and Speech Therapists — restore mobility, function, communication, and swallowing
- Registered Dietitian — plans nutrition and therapeutic diets
- Social Worker — supports psychosocial needs, discharge planning, and resources
- Activities Director — plans recreational and social programming
- Housekeeping, Laundry, Dietary, and Maintenance — support a safe, clean environment

The care plan

The care plan is the written plan of care developed for each resident by the nurse and the interdisciplinary team. The CNA follows the care plan and reports any information that suggests it needs to change. A CNA never changes a care plan independently, and never ignores it because a resident or family member asks.

LESSON 4: SCOPE OF PRACTICE — WHAT A CNA MAY AND MAY NOT DO

Scope of practice is the set of tasks a nurse aide is legally permitted to perform. In Georgia, the scope is defined by state law and federal regulation. Working outside the scope of practice is illegal, endangers residents, and can result in dismissal, removal from the registry, and legal liability.

Within the CNA scope of practice

- Assisting with activities of daily living
- Measuring and recording vital signs, height, weight, intake, and output
- Basic positioning, transferring, and ambulating residents
- Applying non-sterile dressings only where permitted and after training and delegation
- Assisting with elimination, including bedpan, commode, and incontinence care
- Observing, recording, and reporting
- Range of motion and restorative activities as directed in the care plan
- Postmortem care as assigned

Outside the CNA scope of practice — NEVER perform

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- Administering any medication, including over-the-counter medication, eye drops, or topical cream
- Performing sterile procedures
- Inserting or removing tubes, including catheters and feeding tubes
- Performing an assessment or making a nursing diagnosis
- Interpreting results or giving medical advice to a resident or family
- Telling a resident or family a diagnosis, a prognosis, or the results of a test
- Taking a telephone order from a physician
- Applying or removing restraints without a nurse's direction
- Performing any task you have not been trained on and checked off to do
- Signing for care you did not provide

Delegation

Delegation is the process by which a licensed nurse assigns a task to a nurse aide. Only a licensed nurse may delegate to a CNA. The nurse remains accountable for the delegation decision; the CNA is accountable for performing the task correctly, or for saying so when they cannot.

- Accept a delegated task only if it is within the Georgia CNA scope of practice
- Accept it only if you have been trained and checked off on that skill
- Accept it only if you can perform it safely under the current circumstances
- If any of the three is not true, say so and return the task to the nurse

Remember: It is never insubordination to decline a task that is outside your scope, outside your training, or unsafe. Saying "I have not been checked off on that" protects the resident, the facility, and your certification.

LESSON 5: CHAIN OF COMMAND

The chain of command is the line of authority within the facility. Using it correctly makes sure that concerns reach the person who can act on them, and it protects the CNA by documenting that a concern was reported.

- Step 1 — Report to your supervising nurse (charge nurse)
- Step 2 — If unresolved, report to the nurse supervisor or unit manager
- Step 3 — If unresolved, report to the Director of Nursing
- Step 4 — If unresolved, report to the Administrator
- Step 5 — For suspected abuse, neglect, or an unresolved resident safety concern, report externally as required by Georgia mandated reporter law and facility policy

Report immediately — do not wait for the end of the shift

- Any change in a resident's condition, behavior, or level of function
- A fall or a near fall, whether or not the resident appears injured
- New pain, or pain that has changed in character or severity

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- Bleeding, shortness of breath, chest pain, or a change in level of consciousness
- New skin redness, bruising, swelling, or an open area
- Refusal of care, food, or fluids
- Any suspected abuse, neglect, exploitation, or missing personal property
- Any error you made or witnessed

LESSON 6: PROFESSIONALISM & WORK ETHIC

Professional behavior is part of the job, not an addition to it. Facilities evaluate nurse aides on it, and residents experience it directly.

- Dependability — report to work on time, every scheduled shift, and follow the call-out procedure when you cannot
- Accountability — own your errors, report them immediately, and never falsify documentation
- Appearance — clean uniform, identification badge visible, hair secured, nails short and natural, minimal jewelry, no strong fragrance
- Attitude — courteous and respectful toward residents, families, visitors, and every member of the staff
- Confidentiality — protect resident information under HIPAA at all times, on and off duty, including on social media
- Teamwork — help coworkers, answer call lights that are not your assignment, and communicate at handoff
- Empathy — treat each resident as an individual with a history, preferences, and dignity
- Growth — accept feedback, ask questions, and pursue continuing education

Behaviors that are not acceptable

- Using a personal cell phone in a resident care area
- Discussing residents where others can hear, or on social media
- Accepting gifts, tips, or money from a resident or family
- Arguing with a resident, a family member, or a coworker in a resident area
- Working while impaired
- Leaving an assignment without notifying the nurse
- Documenting care that was not provided

LESSON 7: CERTIFICATION, THE REGISTRY & CAREER PATHWAYS

Becoming certified in Georgia

- Complete a state-approved nurse aide training program
- Pass the competency evaluation — a written (or oral) knowledge examination and a clinical skills evaluation, administered by Credentia
- Be listed on the Georgia Nurse Aide Registry, administered by Alliant Health Solutions
- Maintain listing by completing a minimum of eight hours of paid nursing-related work within each 24-month period

The registry also records substantiated findings of resident abuse, neglect, mistreatment, and misappropriation of resident property. A person with such a finding is prohibited from working as a nurse aide in a long-term care facility. A finding follows the individual and cannot be removed by changing employers.

Where the CNA role can lead

- Certified Medication Aide
- Restorative aide, unit clerk, or charge aide roles
- Licensed Practical Nurse (LPN)
- Registered Nurse (RN) — ADN or BSN
- Advanced practice nursing, nursing education, and healthcare leadership

LESSON 8: HOW LONG-TERM CARE IS PAID FOR & THE MDS

Nurse aides are not responsible for billing, but understanding how care is funded explains why documentation, assessment timelines, and survey standards matter as much as they do.

Medicare

Medicare is the federal health insurance program for people 65 and older and for certain younger people with disabilities or end-stage renal disease.

- Part A covers hospital care and a limited period of skilled nursing facility care following a qualifying hospital stay
- Part B covers physician services, outpatient care, and durable medical equipment
- Medicare does not pay for long-term custodial care indefinitely — coverage of a skilled nursing stay is time-limited and requires a documented skilled need

Medicaid

Medicaid is a joint federal and state program that pays for medical and long-term care for people with limited income and assets. In Georgia, Medicaid is the largest single payer for nursing home care, and most residents in long-term care are covered by it.

Private pay and long-term care insurance

- Private pay — the resident or family pays directly
- Long-term care insurance — a policy purchased in advance that covers defined services
- Managed care plans — coverage administered through an insurance organization

The Minimum Data Set (MDS)

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The MDS is the standardized federal assessment completed for every resident in a Medicare- or Medicaid-certified facility. It is part of the Resident Assessment Instrument and drives the care plan, the facility's quality measures, and reimbursement.

- Completed on admission, at scheduled intervals, on significant change in condition, and annually
- Covers function, cognition, mood, continence, skin condition, nutrition, and more
- Completed by licensed staff — a nurse aide does not complete the MDS
- Nurse aide documentation of ADLs, intake, output, and behavior feeds directly into MDS coding

Remember: Your daily documentation is the evidence behind the MDS. Charting an ADL as independent when you actually provided extensive assistance produces an inaccurate assessment, an inaccurate care plan, and inaccurate payment. Document what you actually did.

SKILLS PRACTICE ACTIVITY

Role-play the following:

1. Introducing yourself to a new resident and explaining what you will be doing
2. Reporting a change in a resident's condition to the charge nurse using the chain of command
3. Declining a task that is outside your scope of practice, professionally and without arguing

MODULE SUMMARY

In this module, you learned:

- ✓ The role and responsibilities of the certified nursing assistant
- ✓ The members of the interdisciplinary healthcare team
- ✓ The Georgia CNA scope of practice — what you may do and what you must never do
- ✓ Delegation and the three conditions for accepting a delegated task
- ✓ The chain of command and what must be reported immediately
- ✓ Professionalism, dependability, accountability, and confidentiality
- ✓ Certification, the Nurse Aide Registry, and career advancement pathways

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Remember: You are the eyes and ears of the healthcare team. No one spends more time with the resident than you do. What you observe, and how quickly you report it, changes what happens to that resident.

MODULE 2: COMMUNICATION & INTERPERSONAL SKILLS

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Estimated Lesson Time: 45–60 Minutes

Textbook Reference: Nursing Assistant Care: The Basics, 6th Ed. (Hartman) | Ch 2, pp. 21–30; Ch 10, pp. 252, 255

Federal & State Core Curriculum Alignment: Part I.A — Communication and interpersonal skills including stress management and chain of command (Pre-Clinical, 4 hrs)

LESSON TITLE

Communication & Interpersonal Skills for the Nurse Aide

LESSON OVERVIEW

Effective communication is one of the most important skills a nurse aide can develop. Nurse aides communicate with residents, families, nurses, physicians, coworkers, and other healthcare team members every day.

Good communication promotes:

- Patient safety
- Trust and comfort
- Teamwork
- Accurate care delivery
- Respect and dignity

Poor communication can lead to misunderstandings, errors, patient dissatisfaction, and unsafe care.

As a nurse aide, how you speak, listen, and respond matters.

LEARNING OBJECTIVES

By the end of this lesson, learners will be able to:

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1. Define communication and interpersonal skills.
 2. Differentiate verbal and nonverbal communication.
 3. Demonstrate effective communication with residents and families.
 4. Identify barriers to communication.
 5. Explain therapeutic communication techniques.
 6. Recognize inappropriate communication.
 7. Demonstrate professional communication with the healthcare team.
 8. Apply communication skills in difficult situations.
-

LESSON 1: WHAT IS COMMUNICATION?

Definition of Communication

Communication is the exchange of information, feelings, thoughts, and messages between people.

Healthcare communication includes:

Verbal Communication

Words spoken to another person.

Nonverbal Communication

Messages sent without speaking, such as:

- Facial expressions
- Eye contact
- Body language
- Touch
- Tone of voice

Written Communication

Documentation, charting, and reporting information.

Why Communication Matters in Healthcare

Good communication helps nurse aides:

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- ✓ Build trust with residents
- ✓ Prevent misunderstandings
- ✓ Report changes in condition quickly
- ✓ Reduce anxiety and fear
- ✓ Improve quality of care
- ✓ Promote teamwork

Example

A resident states:

“I do not feel right today.”

A nurse aide should never ignore this statement. Instead:

- ✓ Stop and listen
- ✓ Ask simple questions
- ✓ Observe changes
- ✓ Report concerns to the nurse

LESSON 2: VERBAL COMMUNICATION

Effective Verbal Communication

When speaking with residents:

Speak Clearly

Use simple language.

Speak Slowly

Allow time for understanding.

Be Respectful

Always use respectful language.

Address Residents Properly

Use:

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- Mr.
- Mrs.
- Ms.

Unless the resident asks otherwise.

Examples of Respectful Communication

 Poor Communication
“Come on, let’s hurry up.”

 Better Communication
“Mr. Johnson, let me help you get ready. Take your time.”

Active Listening

Listening is one of the most important communication skills.

Good listeners:

- Maintain eye contact
- Avoid interrupting
- Show empathy
- Ask clarifying questions
- Pay attention

Example

Resident:

“I miss my family.”

Poor response:

 “Okay.”

Better response:

 “I’m sorry you’re feeling lonely. Would you like to talk about it?”

INTERPERSONAL SKILLS

What Are Interpersonal Skills?

Interpersonal skills are the ways people interact with one another.

Good interpersonal skills include:

Compassion

Showing kindness and concern.

Patience

Remaining calm.

Respect

Treating everyone with dignity.

Empathy

Understanding another person's feelings.

Professionalism

Maintaining respectful boundaries.

LESSON 3: NONVERBAL COMMUNICATION

Residents pay attention to body language.

Examples of nonverbal communication:

Positive Examples

- ✓ Smiling
- ✓ Gentle tone
- ✓ Eye contact
- ✓ Calm posture

Negative Examples

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- ✗ Eye rolling
 - ✗ Crossing arms
 - ✗ Looking angry
 - ✗ Ignoring residents
-

Personal Space

Respect a resident's privacy and personal space.

Always:

- Knock before entering
- Introduce yourself
- Explain care before touching

Example:

“Hello Mrs. Smith, my name is Sarah, and I am here to help you get ready for breakfast.”

LESSON 4: BARRIERS TO COMMUNICATION

Barriers are things that make communication difficult.

Hearing Problems

Speak clearly—not loudly.

Vision Problems

Describe surroundings.

Language Differences

Use simple words and interpreter assistance when available.

Dementia or Confusion

Use:

- Short sentences
- Calm tone

- Repetition

Emotional Stress

Fear, pain, anxiety, or grief may affect communication.

Communication With Residents With Dementia

DO:

- ☒ Speak calmly
- ☒ Use short instructions
- ☒ Maintain eye contact
- ☒ Reassure the resident

DO NOT:

- ☒ Argue
- ☒ Correct harshly
- ☒ Rush the resident

Example:

Resident:

“I need to go pick up my children.”

Poor response:

- ☒ “Your children are adults!”

Better response:

- ☒ “Tell me about your children.”
-

LESSON 5: COMMUNICATION WITH THE HEALTHCARE TEAM

CNAs must report changes to nurses immediately.

Report:

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- Pain
 - Fever
 - Shortness of breath
 - Falls
 - Skin changes
 - Confusion
 - Refusing meals
 - Change in behavior
-

SBAR Communication

S – Situation

What is happening?

B – Background

Relevant information

A – Assessment

What you observed

R – Recommendation

What action is needed

Example

“Mr. Jones appears short of breath. He was fine earlier but now looks pale and is breathing fast.”

SKILLS PRACTICE ACTIVITY

Role-play the following:

1. Greeting a resident respectfully
 2. Reporting a concern to the nurse
 3. Communicating with an upset resident
-

LESSON SUMMARY

In this lesson, you learned:

- Communication basics
- Verbal and nonverbal communication
- Interpersonal skills
- Therapeutic communication
- Barriers to communication
- Communication with dementia patients
- Reporting changes in condition
- Team communication

Remember: Great CNAs are great communicators. Communication builds trust, improves safety, and promotes compassionate care.

MODULE 3: INFECTION CONTROL

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Estimated Completion Time: 2–3 Hours

Textbook Reference: Nursing Assistant Care: The Basics, 6th Ed. (Hartman) | Ch 2, pp. 46–62

Federal & State Core Curriculum Alignment: Part I — Infection Control (Pre-Clinical, 4 hrs)

MODULE TITLE

Infection Control & Prevention for the Nurse Aide

MODULE OVERVIEW

Infection prevention and control are among the most important responsibilities of a nurse aide. Residents and patients in healthcare settings may have weakened immune systems, chronic illnesses, wounds, or medical conditions that place them at greater risk for infection.

As a nurse aide, your actions can either help stop the spread of infection or contribute to illness transmission.

This module teaches nurse aides how to prevent infections through:

- Proper hand hygiene
- Standard precautions
- Personal protective equipment (PPE)
- Transmission-based precautions
- Environmental cleaning
- Safe handling of equipment and linens
- Prevention of healthcare-associated infections (HAIs)

Preventing infection saves lives.

LEARNING OBJECTIVES

By the end of this module, learners will be able to:

4. Define infection and infection control.
 5. Explain how infections spread.
 6. Identify the chain of infection.
 7. Describe methods to break the chain of infection.
 8. Demonstrate proper hand hygiene.
 9. Identify when and how to use PPE.
 10. Differentiate standard and transmission-based precautions.
 11. Demonstrate proper cleaning and disposal techniques.
 12. Apply infection control practices in resident care situations.
-

LESSON 1: INTRODUCTION TO INFECTION CONTROL

What Is Infection?

An infection occurs when harmful microorganisms (germs) enter the body and multiply, causing illness.

Common microorganisms include:

Bacteria

Tiny organisms that may cause infections.

Examples:

- Pneumonia
- Urinary tract infection (UTI)

Viruses

Organisms that require a host to grow.

Examples:

- Influenza (flu)
- COVID-19
- Common cold

Fungi

Can cause infections of the skin or body.

Example:

- Yeast infection

Parasites

Organisms that live on or inside the body.

Example:

- Scabies
-

Why Infection Control Matters

Healthcare-associated infections can cause:

- Serious illness
- Longer hospital stays
- Increased healthcare costs
- Death

Nurse aides play a major role in preventing infection.

LESSON 2: THE CHAIN OF INFECTION

What Is the Chain of Infection?

Infections spread through a process called the **chain of infection**.

There are **6 links** in the chain.

1. Infectious Agent

The germ causing disease.

Examples:

- Bacteria
- Viruses
- Fungi

2. Reservoir

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Where germs live.

Examples:

- Humans
- Animals
- Food
- Water
- Equipment

3. Portal of Exit

How germs leave the body.

Examples:

- Coughing
- Sneezing
- Blood
- Urine
- Drainage

4. Mode of Transmission

How germs spread.

Examples:

- Touch
- Air
- Droplets
- Contaminated surfaces

5. Portal of Entry

How germs enter another person.

Examples:

- Mouth
- Nose
- Broken skin
- Catheters

6. Susceptible Host

A person likely to get sick.

Examples:

- Elderly residents
 - People with chronic illness
 - Immunocompromised patients
-

Breaking the Chain of Infection

Healthcare workers can stop infection by:

- ✓ Washing hands
 - ✓ Wearing PPE
 - ✓ Cleaning equipment
 - ✓ Proper disposal of waste
 - ✓ Covering coughs/sneezes
 - ✓ Staying home when sick
-

LESSON 3: HAND HYGIENE

Handwashing Saves Lives

Hand hygiene is the **single most important way to prevent infection.**

Wash Hands:

- ✓ Before touching a resident
 - ✓ After touching a resident
 - ✓ Before eating
 - ✓ After using the restroom
 - ✓ After removing gloves
 - ✓ After handling body fluids
 - ✓ After coughing or sneezing
-

Proper Handwashing Steps

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Step 1

Wet hands with warm water.

Step 2

Apply soap.

Step 3

Rub hands for **at least 20 seconds**.

Clean:

- Front of hands
- Back of hands
- Between fingers
- Fingernails
- Wrists

Step 4

Rinse thoroughly.

Step 5

Dry hands completely.

Step 6

Use towel to turn off faucet.

Alcohol-Based Hand Sanitizer

Can be used when hands are **not visibly dirty**.

Rub hands until dry.

VIDEO/DEMONSTRATION ACTIVITY

Watch instructor demonstration: Proper Handwashing

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Students must demonstrate:

- ✓ Proper timing
 - ✓ Proper technique
 - ✓ Full hand coverage
-

LESSON 4: PERSONAL PROTECTIVE EQUIPMENT (PPE)

What Is PPE?

PPE protects healthcare workers and residents from infection.

Examples include:

Gloves

Protect hands.

Use when:

- Touching blood
 - Body fluids
 - Broken skin
 - Dirty linens
-

Gowns

Protect clothing.

Masks

Protect mouth and nose.

Face Shields/Goggles

Protect eyes.

Proper Order for Putting On PPE

1. Gown
 2. Mask
 3. Goggles/Face Shield
 4. Gloves
-

Removing PPE Safely

Remove carefully to avoid contamination.

Always wash hands afterward.

SKILLS PRACTICE

Demonstrate:

- ✓ Putting on gloves
 - ✓ Removing gloves safely
 - ✓ Handwashing after glove removal
-

LESSON 5: STANDARD PRECAUTIONS

What Are Standard Precautions?

Treat **all blood and body fluids as potentially infectious.**

Use precautions with every resident.

Examples:

- Gloves
 - Hand hygiene
 - Safe disposal
 - Cleaning equipment
-

Body Fluids That May Spread Infection

- Blood
 - Urine
 - Stool
 - Vomit
 - Saliva
 - Drainage
-

LESSON 6: TRANSMISSION-BASED PRECAUTIONS

Some infections require extra precautions.

Contact Precautions

Used for infections spread by touch.

Examples:

- MRSA
- C. difficile

Wear:

- ☒ Gloves
 - ☒ Gown
-

Droplet Precautions

Spread by coughing or sneezing.

Examples:

- Flu
- COVID-19

Wear:

- ☒ Mask
-

Airborne Precautions

Spread through tiny particles in the air.

Examples:

- Tuberculosis (TB)

Wear:

- ☒ Special mask (N95)
-

LESSON 7: ENVIRONMENTAL CLEANING & LINEN HANDLING

Dirty Linen Handling

Do NOT shake linens.

Carry linens away from uniform.

Place in proper laundry bag.

Cleaning Resident Areas

Clean:

- Bedside tables
- Equipment
- High-touch surfaces

Prevent cross-contamination.

Sharps Safety

Never touch used needles.

Never recap needles.

Report injuries immediately.

MODULE SUMMARY

In this module, you learned:

- ✓ How infections spread
- ✓ The chain of infection
- ✓ Hand hygiene
- ✓ PPE use
- ✓ Standard precautions
- ✓ Transmission-based precautions
- ✓ Environmental cleaning
- ✓ Linen handling and safety

Remember: Infection control protects **you, your residents, coworkers, and families.** Great CNAs make infection prevention a daily habit.

MODULE 4: SAFETY & EMERGENCY PROCEDURES

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Estimated Completion Time: 2–3 Hours

Textbook Reference: Nursing Assistant Care: The Basics, 6th Ed. (Hartman) | Ch 2, pp. 31–45; Ch 6, p. 162

Federal & State Core Curriculum Alignment: Part I.B — Safety/Emergency/FBAO/Body Mechanics (Pre-Clinical, 4 hrs)

MODULE TITLE

Safety & Emergency Procedures for the Nurse Aide

MODULE OVERVIEW

Safety is one of the most important responsibilities of a nurse aide. Residents in healthcare settings may be elderly, weak, confused, physically disabled, or at risk for injury. Nurse aides must create and maintain a safe environment while recognizing emergencies and responding appropriately.

This module teaches nurse aides how to:

- Promote resident safety
- Prevent falls and injuries
- Identify safety hazards
- Use proper body mechanics
- Respond to emergencies
- Follow fire safety procedures
- Recognize disasters and evacuation procedures
- Report accidents and incidents

Patient safety is everyone's responsibility.

LEARNING OBJECTIVES

By the end of this module, learners will be able to:

1. Explain the importance of safety in healthcare.
 2. Identify common safety hazards.
 3. Demonstrate fall prevention strategies.
 4. Apply proper body mechanics.
 5. Describe emergency response procedures.
 6. Explain fire safety and evacuation protocols.
 7. Recognize signs of medical emergencies.
 8. Demonstrate accident prevention techniques.
 9. Describe reporting requirements after incidents.
-

LESSON 1: INTRODUCTION TO SAFETY

What Is Safety?

Safety means protecting residents, coworkers, and yourself from injury or harm.

A safe healthcare environment helps prevent:

- Falls
- Burns
- Poisoning
- Equipment injuries
- Infection
- Medical emergencies

As a nurse aide, you help maintain safety every day.

Why Safety Matters

Residents may have:

- Weakness
- Memory problems
- Confusion
- Limited mobility
- Poor balance
- Vision impairment

Even small mistakes can lead to serious injuries.

Example

Leaving water spilled on the floor may result in a resident fall.

LESSON 2: FALL PREVENTION

Falls Are a Major Safety Concern

Falls can cause:

- Broken bones
 - Head injuries
 - Disability
 - Fear of movement
 - Death
-

Common Fall Risks

Residents may fall because of:

Weakness

Difficulty walking or standing.

Dizziness

Medication side effects.

Confusion

Forgetfulness or dementia.

Poor Vision

Difficulty seeing hazards.

Unsafe Environment

Clutter, wet floors, poor lighting.

CNA Fall Prevention Strategies

Always:

- ✓ Keep call light within reach
 - ✓ Keep bed in lowest position
 - ✓ Lock wheelchair and bed brakes
 - ✓ Remove clutter
 - ✓ Use non-slip footwear
 - ✓ Assist residents when walking
 - ✓ Follow transfer precautions
-

NEVER:

- ✗ Leave confused residents unattended when unsafe
 - ✗ Ignore fall precautions
 - ✗ Rush residents
-

LESSON 3: BODY MECHANICS & SAFE TRANSFERS

Why Body Mechanics Matter

Poor lifting techniques can cause injuries to:

- Back
- Neck
- Shoulders
- Knees

Proper body mechanics protect both the CNA and resident.

Proper Body Mechanics

Lift With Legs

Never your back.

Keep Feet Apart

Maintain balance.

Bend Knees

Avoid bending from waist.

Keep Object Close

Reduces strain.

Ask for Help

When resident is heavy or unstable.

Transfer Safety

Always:

- ☒ Explain procedure
 - ☒ Lock wheelchair brakes
 - ☒ Use gait belt if required
 - ☒ Follow care plan
 - ☒ Ask for assistance when needed
-

SKILLS ACTIVITY

Demonstrate:

1. Proper transfer setup
 2. Wheelchair safety
 3. Correct body mechanics
-

LESSON 4: FIRE SAFETY

Fire Emergencies in Healthcare

Healthcare facilities follow fire safety procedures to protect residents.

Common Causes of Fire

- Electrical problems
 - Smoking materials
 - Kitchen accidents
 - Oxygen misuse
-

RACE Method

R – Rescue

Move residents from danger.

A – Alarm

Pull fire alarm and notify staff.

C – Contain

Close doors and windows.

E – Extinguish/Evacuate

Use extinguisher if trained or evacuate.

PASS Method (Fire Extinguisher)

P – Pull pin

A – Aim at base of fire

S – Squeeze handle

S – Sweep side to side

LESSON 5: DISASTER & EMERGENCY PREPAREDNESS

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Healthcare emergencies may include:

- Tornadoes
 - Hurricanes
 - Power outages
 - Active threats
 - Chemical spills
 - Severe weather
-

CNA Responsibilities During Emergencies

Stay calm.

Follow facility policy.

Protect residents.

Follow chain of command.

Never abandon residents.

Emergency Equipment

Know location of:

- Fire exits
 - Emergency carts
 - Oxygen shut-off
 - First aid kits
 - Emergency phones
-

LESSON 6: MEDICAL EMERGENCIES

CNAs must recognize emergencies and report them immediately.

Signs of Emergency

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Chest Pain

Could indicate heart attack.

Difficulty Breathing

Medical emergency.

Sudden Confusion

Possible stroke or infection.

Unresponsiveness

Immediate emergency.

Severe Bleeding

Needs urgent attention.

Fall With Injury

Must report immediately.

Stroke Warning Signs (FAST)

F – Face drooping

A – Arm weakness

S – Speech difficulty

T – Time to call for help

Choking Emergency

Signs include:

- Cannot speak
- Hands on throat
- Difficulty breathing

Report immediately and seek trained assistance.

LESSON 7: ACCIDENT PREVENTION & REPORTING

Preventing Accidents

Always:

- ✓ Keep floors dry
 - ✓ Use safety devices properly
 - ✓ Follow transfer plans
 - ✓ Keep walkways clear
-

Incident Reporting

Report immediately:

- Falls
 - Injuries
 - Unsafe equipment
 - Resident changes
 - Accidents
-

Documentation

Be:

- ✓ Accurate
- ✓ Honest
- ✓ Timely
- ✓ Objective

Never guess or change facts.

ROLE-PLAY ACTIVITY

Practice:

1. Assisting a resident safely to chair
2. Reporting a medical emergency

MODULE SUMMARY

In this module, you learned:

- ✓ Resident safety principles
- ✓ Fall prevention
- ✓ Proper body mechanics
- ✓ Safe transfers
- ✓ Fire safety (RACE & PASS)
- ✓ Emergency preparedness
- ✓ Medical emergency recognition
- ✓ Accident prevention and reporting

Remember: A safe CNA is a successful CNA. Preventing accidents and responding quickly to emergencies protects lives and promotes quality care.

MODULE 5: RESIDENTS' RIGHTS & DIGNITY

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Estimated Completion Time: 2–3 Hours

Textbook Reference: Nursing Assistant Care: The Basics, 6th Ed. (Hartman) | Ch 1, pp. 10–18; Ch 3, pp. 65–66; Ch 6, p. 128

Federal & State Core Curriculum Alignment: Part I.C — Promoting Residents' Independence | Part I.D — Respecting Residents' Rights (Pre-Clinical, 4 hrs) | Part VII — Residents' Rights

MODULE TITLE

Residents' Rights, Privacy, Independence & Dignity

MODULE OVERVIEW

Every resident has the right to be treated with dignity, respect, compassion, and fairness. Nurse aides play a major role in protecting resident rights while providing safe, ethical, and patient-centered care.

Residents are individuals with personal values, beliefs, cultures, and preferences. Illness, disability, or aging does not remove a person's rights or dignity.

This module teaches nurse aides how to:

- Protect residents' rights
- Promote dignity and independence
- Maintain privacy and confidentiality
- Respect cultural differences
- Prevent abuse, neglect, and mistreatment
- Support informed choice and autonomy
- Communicate respectfully

Residents deserve care that honors who they are as individuals.

LEARNING OBJECTIVES

By the end of this module, learners will be able to:

1. Define residents' rights.
 2. Explain the importance of dignity and respect.
 3. Identify ways to protect privacy and confidentiality.
 4. Demonstrate respect for cultural differences.
 5. Promote resident independence.
 6. Identify abuse, neglect, and exploitation.
 7. Explain mandatory reporting responsibilities.
 8. Apply ethical care practices in daily resident interactions.
-

LESSON 1: WHAT ARE RESIDENTS' RIGHTS?

Definition of Residents' Rights

Residents' rights are legal and ethical protections that ensure residents are treated fairly, safely, and respectfully.

Residents have the right to:

- ✓ Be treated with dignity and respect
 - ✓ Make decisions about their care
 - ✓ Privacy and confidentiality
 - ✓ Be free from abuse and neglect
 - ✓ Participate in care planning
 - ✓ Refuse treatment (within policy)
 - ✓ Voice complaints without retaliation
 - ✓ Safe and quality care
-

Why Residents' Rights Matter

Residents often depend on caregivers for assistance.

Protecting rights helps:

- Preserve dignity
 - Build trust
 - Promote independence
 - Improve emotional well-being
 - Prevent mistreatment
-

Example

A resident refuses breakfast.

Incorrect response:

✗ “You have to eat!”

Better response:

✓ “Would you like something different or prefer to eat later?”

LESSON 2: PROMOTING DIGNITY

What Is Dignity?

Dignity means treating residents as valuable, respected individuals.

Dignity includes:

Respect

Treating residents politely.

Compassion

Showing kindness and concern.

Choice

Allowing residents to make decisions when possible.

Individuality

Respecting personal preferences.

Ways CNAs Promote Dignity

Always:

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- ✓ Knock before entering
 - ✓ Address residents respectfully
 - ✓ Explain care before providing it
 - ✓ Cover residents during bathing
 - ✓ Protect privacy during personal care
 - ✓ Speak respectfully
-

Avoid Infantilizing Residents

Residents are adults.

Do NOT:

- ✗ Use baby talk
- ✗ Call residents “honey” or “sweetie” unless preferred
- ✗ Speak down to residents

Instead:

- ✓ Ask how they prefer to be addressed.

Example:

“How would you like me to address you?”

LESSON 3: PRIVACY & CONFIDENTIALITY

Resident Privacy

Residents have a right to privacy during:

- Bathing
 - Dressing
 - Toileting
 - Medical care
 - Phone calls
 - Visits
-

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Ways to Protect Privacy

Always:

- ☒ Knock before entering
 - ☒ Close curtains or doors
 - ☒ Cover residents during care
 - ☒ Speak quietly about medical information
-

Confidentiality & HIPAA

Resident information is private.

Never discuss:

- ☒ Medical conditions in hallways
- ☒ Resident information on social media
- ☒ Private details with unauthorized people

Only share information with:

- ☒ Healthcare team members involved in care
-

Example

Incorrect:

- ☒ Discussing a resident's diagnosis in the cafeteria.

Correct:

- ☒ Sharing necessary information privately with the nurse.
-

LESSON 4: PROMOTING INDEPENDENCE

Why Independence Matters

Residents often feel loss of control.

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Encouraging independence:

- Improves self-esteem
 - Maintains abilities
 - Promotes confidence
 - Supports quality of life
-

How CNAs Promote Independence

Instead of doing everything for residents:

Encourage them to:

- ☒ Feed themselves
 - ☒ Brush hair
 - ☒ Choose clothing
 - ☒ Participate in care
-

Example

Instead of:

☒ Feeding resident immediately

Try:

- ☒ “Would you like to try eating first? I can help if needed.”
-

Safety & Independence Balance

Encourage independence safely.

Always follow:

- Care plan
 - Nurse instructions
 - Fall precautions
-

LESSON 5: CULTURAL AWARENESS & RESPECT

Cultural Sensitivity

Residents may have different:

- Languages
- Religions
- Food preferences
- Family traditions
- Healthcare beliefs

CNAs should provide respectful care to all residents.

Examples of Cultural Respect

- ✓ Respect religious practices
 - ✓ Honor food preferences when possible
 - ✓ Ask questions respectfully
 - ✓ Avoid stereotypes
-

Example

Resident requests prayer before meals.

Appropriate response:

- ✓ Respect the resident's preference.
-

LESSON 6: ABUSE, NEGLECT & EXPLOITATION

Types of Abuse

Physical Abuse

Hitting, pushing, rough handling.

Emotional Abuse

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Yelling, threatening, humiliation.

Verbal Abuse

Insults or disrespectful language.

Sexual Abuse

Any unwanted sexual behavior.

Financial Exploitation

Stealing money or belongings.

Neglect

Failing to provide care.

Warning Signs of Abuse

- Bruises
 - Fearfulness
 - Sudden behavior changes
 - Poor hygiene
 - Weight loss
 - Withdrawal
-

CNA Responsibilities

CNAs are **mandatory reporters**.

If abuse is suspected:

- ☒ Report immediately to nurse or supervisor.

Do NOT:

- ☒ Ignore signs
- ☒ Investigate on your own
- ☒ Promise secrecy

LESSON 7: ETHICAL CARE & PROFESSIONAL BOUNDARIES

Ethics in Healthcare

Ethics means doing what is right.

CNAs should:

- ✓ Be honest
 - ✓ Respect residents
 - ✓ Maintain boundaries
 - ✓ Protect confidentiality
-

Professional Boundaries

Avoid:

- ✗ Accepting expensive gifts
- ✗ Sharing personal problems excessively
- ✗ Becoming overly involved

Maintain professional relationships.

LESSON 8: RESTRAINTS & THE RESTRAINT-FREE ENVIRONMENT

Every resident has the right to be free from restraint imposed for discipline or staff convenience. Federal law strongly favors a restraint-free environment, and restraint use is tightly restricted.

What is a restraint?

- Physical restraint — any manual method, device, material, or equipment attached to or near the body that the resident cannot remove easily and that restricts freedom of movement or normal access to one's own body
- Chemical restraint — a medication used to control behavior or restrict movement that is not required to treat a medical condition
- Examples include vest and belt devices, wrist and ankle devices, certain lap trays and full side rails, and tucking a sheet so tightly the resident cannot move

Legal restrictions

- A restraint requires a physician's order that states the reason, the type, and the duration
- A restraint may never be used for discipline, punishment, or staff convenience
- Less restrictive alternatives must be tried and documented first
- A nurse aide never applies or removes a restraint without the direction of a licensed nurse
- Applying a restraint without an order may constitute false imprisonment

Risks of restraint use

- Pressure injuries, muscle weakness, contracture, and loss of mobility
- Incontinence, constipation, and reduced appetite
- Pneumonia and blood clots
- Agitation, depression, withdrawal, and loss of dignity
- Serious injury or death from strangulation or entrapment

Restraint alternatives

- Identify and address the underlying need — pain, hunger, thirst, toileting, boredom, fear
- Increase supervision and check on the resident more frequently
- Lower the bed and use a floor mat; use bed and chair alarms where permitted
- Provide activity, exercise, and companionship
- Ensure eyeglasses, hearing aids, and assistive devices are in use and working
- Answer call lights promptly and keep needed items within reach

When a restraint is in use

- Check the resident at least every 15 minutes
- Release the restraint at least every 2 hours for at least 10 minutes
- Offer toileting, fluids, repositioning, and range of motion at each release
- Check circulation, skin condition, and correct positioning of the device
- Report any skin change, distress, or difficulty breathing to the nurse immediately
- Document every check, release, and care measure provided

Remember: The safest restraint is the one that is never applied. Look for the unmet need behind the behavior before anyone reaches for a device.

SKILLS PRACTICE ACTIVITY

Role-play:

1. Respectful resident greeting
2. Protecting privacy during care
3. Encouraging independence
4. Reporting suspected abuse

MODULE SUMMARY

In this module, you learned:

- ✓ Residents' rights
- ✓ Dignity and respect
- ✓ Privacy and confidentiality
- ✓ HIPAA basics
- ✓ Promoting independence
- ✓ Cultural sensitivity
- ✓ Abuse and neglect prevention
- ✓ Mandatory reporting
- ✓ Professional boundaries

Remember: Every resident deserves to feel respected, valued, safe, and heard. Protecting dignity and rights is one of the most important responsibilities of a CNA.

MODULE 6: ABUSE, NEGLECT & MISAPPROPRIATION

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Estimated Completion Time: 2–3 Hours

Textbook Reference: Nursing Assistant Care: The Basics, 6th Ed. (Hartman) | Ch 1, pp. 10–20

Federal & State Core Curriculum Alignment: Part VII.F — Freedom from Abuse, Mistreatment, and Neglect; Reporting

MODULE TITLE

Abuse, Neglect, Exploitation & Misappropriation Prevention

MODULE OVERVIEW

Residents in healthcare settings have the right to be safe, respected, and protected from harm. Nurse aides have a legal, ethical, and professional responsibility to recognize, prevent, and report abuse, neglect, exploitation, and misappropriation of resident property.

Failure to report suspected abuse may place residents at risk and may result in disciplinary action, legal consequences, or loss of certification.

This module teaches nurse aides how to:

- Recognize abuse and neglect
- Identify signs of mistreatment
- Understand misappropriation of resident property
- Report suspected abuse immediately
- Protect vulnerable residents
- Maintain professional boundaries
- Follow mandatory reporting requirements

Abuse prevention is everyone's responsibility.

LEARNING OBJECTIVES

By the end of this module, learners will be able to:

1. Define abuse, neglect, exploitation, and misappropriation.
2. Identify different forms of abuse.
3. Recognize warning signs of abuse and neglect.
4. Explain CNA reporting responsibilities.
5. Describe resident protections.
6. Apply prevention strategies.
7. Demonstrate professional boundaries.
8. Explain consequences of failing to report abuse.

LESSON 1: UNDERSTANDING ABUSE

What Is Abuse?

Abuse is any intentional act that causes harm, pain, distress, fear, or injury to a resident.

Abuse can be:

- Physical
- Emotional
- Verbal
- Sexual
- Financial
- Psychological

Residents may be vulnerable because of:

- Age
- Illness
- Disability
- Cognitive impairment
- Dependence on caregivers

Residents Have the Right To:

- ✓ Be safe
- ✓ Be treated respectfully
- ✓ Live free from abuse
- ✓ Report concerns without fear

LESSON 2: TYPES OF ABUSE

1. Physical Abuse

Physical abuse involves causing bodily harm.

Examples include:

- ✗ Hitting
- ✗ Slapping
- ✗ Pinching
- ✗ Pushing
- ✗ Rough handling

Warning Signs

- Bruises
 - Fractures
 - Fear of caregivers
 - Unexplained injuries
-

2. Verbal Abuse

Using words to hurt or intimidate a resident.

Examples:

- ✗ Yelling
- ✗ Threatening
- ✗ Humiliating
- ✗ Mocking

Example

Incorrect:

“You are too slow!”

Correct:

“Take your time. I’m here to help.”

3. Emotional/Psychological Abuse

Behavior causing emotional pain or fear.

Examples:

- ✗ Ignoring residents
- ✗ Isolation
- ✗ Threats
- ✗ Intimidation

Warning Signs

- Anxiety
 - Withdrawal
 - Depression
 - Fearfulness
-

4. Sexual Abuse

Any unwanted sexual contact or behavior.

Examples:

- ✗ Inappropriate touching
- ✗ Sexual comments
- ✗ Forced activity

Warning Signs

- Bruising
 - Fear
 - Sudden behavior changes
-

5. Financial Exploitation

Improper use of a resident's money or property.

Examples:

- ✗ Taking money
 - ✗ Using credit cards
 - ✗ Stealing belongings
 - ✗ Coercing gifts
-

LESSON 3: WHAT IS NEGLECT?

Definition of Neglect

Neglect occurs when a caregiver fails to provide necessary care.

Examples include:

- ✗ Not helping with toileting
 - ✗ Ignoring call lights
 - ✗ Failure to reposition resident
 - ✗ Withholding food or water
 - ✗ Poor hygiene assistance
-

Consequences of Neglect

Neglect may lead to:

- Falls
 - Skin breakdown
 - Pressure ulcers
 - Malnutrition
 - Emotional distress
 - Infection
-

Example

A resident lies in wet clothing for several hours.

This is:

- ☒ Neglect
-

LESSON 4: MISAPPROPRIATION OF RESIDENT PROPERTY

What Is Misappropriation?

Misappropriation means the intentional misuse, theft, or wrongful taking of a resident's property or belongings.

Examples include:

- ☒ Taking money
 - ☒ Using resident credit card
 - ☒ Borrowing jewelry
 - ☒ Taking food or belongings without permission
-

Resident Property Includes

- Money
 - Clothing
 - Jewelry
 - Phones
 - Personal items
 - Medical equipment
-

Prevention Tips

Always:

- ☒ Respect resident belongings
- ☒ Document valuables per facility policy
- ☒ Report missing items immediately

Never:

- ✗ Borrow money
 - ✗ Accept valuables
 - ✗ Take personal property
-

LESSON 5: REPORTING ABUSE & NEGLECT

CNAs Are Mandatory Reporters

A CNA must report:

- ✓ Suspected abuse
 - ✓ Neglect
 - ✓ Exploitation
 - ✓ Missing property
 - ✓ Resident complaints
-

What To Do If You Suspect Abuse

Step 1

Ensure resident safety.

Step 2

Report immediately to:

- Nurse
- Supervisor
- Administrator

Step 3

Document facts objectively.

What NOT To Do

- ✗ Ignore warning signs
- ✗ Investigate independently

- ✗ Confront accused staff aggressively
 - ✗ Promise secrecy
-

Important Rule

You do NOT need proof to report suspicion.

You only need reasonable concern.

LESSON 6: PROFESSIONAL BOUNDARIES & PREVENTION

Maintaining Professional Boundaries

CNAs should:

- ✓ Be respectful
- ✓ Protect residents
- ✓ Stay professional

Avoid:

- ✗ Accepting expensive gifts
 - ✗ Sharing personal financial issues
 - ✗ Borrowing money
 - ✗ Favoritism
-

Preventing Abuse

Healthcare workers prevent abuse by:

- ✓ Treating residents respectfully
 - ✓ Managing stress professionally
 - ✓ Following care plans
 - ✓ Reporting concerns immediately
 - ✓ Using teamwork
-

Stress & Burnout Awareness

Stress may increase risk for poor care.

Healthy coping strategies include:

- Taking breaks
- Asking for support
- Managing workload
- Professional communication

SKILLS PRACTICE ACTIVITY

Role-play:

1. Reporting suspected neglect
2. Responding to resident complaint
3. Maintaining respectful communication
4. Protecting resident property

MODULE SUMMARY

In this module, you learned:

- ✓ Types of abuse
- ✓ Warning signs of mistreatment
- ✓ Neglect prevention
- ✓ Misappropriation of property
- ✓ Mandatory reporting responsibilities
- ✓ Professional boundaries
- ✓ Prevention strategies

Remember: CNAs are resident advocates. If you see something concerning, speak up. Reporting abuse, neglect, or misappropriation can protect a resident from serious harm.

MODULE 7: BODY SYSTEMS & DISEASE PROCESSES

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Estimated Completion Time: 3–4 Hours

Textbook Reference: Nursing Assistant Care: The Basics, 6th Ed. (Hartman) | Ch 4, pp. 83–114; Ch 5, pp. 115–127

Federal & State Core Curriculum Alignment: Part II.D — Recognizing Abnormal Changes in Body Function and the Importance of Reporting Such Changes to a Supervisor

MODULE TITLE

Understanding Body Systems & Common Disease Processes

MODULE OVERVIEW

Nurse aides care for residents with many medical conditions and chronic illnesses. Although CNAs do not diagnose or treat diseases, it is important to understand how the body works and recognize common signs and symptoms of illness.

Understanding body systems helps nurse aides:

- Recognize changes in condition
- Provide safer care
- Understand resident limitations
- Improve communication with nurses
- Report concerns promptly

This module introduces major body systems and common diseases seen in long-term care, hospitals, rehabilitation, and home care settings.

Remember: CNAs observe and report changes—they do not diagnose.

LEARNING OBJECTIVES

By the end of this module, learners will be able to:

1. Identify major body systems.

2. Explain the function of each body system.
 3. Recognize common diseases affecting body systems.
 4. Identify signs and symptoms that should be reported.
 5. Explain how disease affects resident care.
 6. Demonstrate understanding of CNA observation responsibilities.
 7. Recognize emergencies related to body system changes.
-

LESSON 1: INTRODUCTION TO BODY SYSTEMS

What Are Body Systems?

The human body is made up of systems that work together to keep a person healthy and functioning.

Major body systems include:

1. Musculoskeletal System
 2. Nervous System
 3. Cardiovascular System
 4. Respiratory System
 5. Digestive System
 6. Urinary System
 7. Endocrine System
 8. Integumentary System
 9. Immune/Lymphatic System
 10. Reproductive System
-

CNA Role in Disease Care

CNAs:

- ✓ Observe residents
- ✓ Report changes promptly
- ✓ Follow care plans
- ✓ Assist with activities of daily living (ADLs)

CNAs do NOT:

- ✗ Diagnose illness
 - ✗ Prescribe medication
 - ✗ Change treatment plans
-

LESSON 2: MUSCULOSKELETAL SYSTEM

Function

The musculoskeletal system includes:

- Bones
- Muscles
- Joints
- Ligaments

This system helps the body:

- Move
 - Maintain posture
 - Protect organs
-

Common Diseases

Arthritis

Inflammation of joints causing pain and stiffness.

Signs:

- Joint pain
- Swelling
- Difficulty moving

Osteoporosis

Weak bones that break easily.

Risk:

⚠ Increased falls/fractures

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CNA Care

- ✓ Assist with mobility
 - ✓ Encourage safe movement
 - ✓ Prevent falls
 - ✓ Report pain changes
-

LESSON 3: NERVOUS SYSTEM

Function

Controls:

- Brain
- Spinal cord
- Nerves

Responsible for:

- Thinking
 - Movement
 - Memory
 - Sensation
-

Common Conditions

Stroke (CVA)

Occurs when blood flow to brain is interrupted.

FAST Warning Signs

F – Face drooping

A – Arm weakness

S – Speech difficulty

T – Time to call for help

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Dementia

Progressive memory decline.

Signs:

- Confusion
 - Forgetfulness
 - Wandering
-

Parkinson's Disease

Affects movement.

Signs:

- Tremors
 - Slow movement
 - Balance issues
-

CNA Responsibilities

- ✓ Maintain safety
 - ✓ Reorient calmly
 - ✓ Report sudden confusion
 - ✓ Watch for falls
-

LESSON 4: CARDIOVASCULAR SYSTEM

Function

Heart and blood vessels circulate oxygen-rich blood throughout the body.

Common Diseases

Hypertension (High Blood Pressure)

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Often called “silent killer.”

Signs may include:

- Headache
 - Dizziness
-

Heart Failure (CHF)

Heart cannot pump effectively.

Signs:

- Swelling
 - Shortness of breath
 - Fatigue
-

Heart Attack

Emergency condition.

Symptoms:

- ⚠ Chest pain
 - ⚠ Sweating
 - ⚠ Nausea
 - ⚠ Shortness of breath
-

CNA Responsibilities

Report immediately:

- ✅ Chest pain
 - ✅ Difficulty breathing
 - ✅ Swelling changes
-

LESSON 5: RESPIRATORY SYSTEM

Function

Helps body breathe and receive oxygen.

Organs include:

- Lungs
 - Airways
-

Common Diseases

COPD

Chronic lung disease.

Signs:

- Shortness of breath
 - Wheezing
-

Pneumonia

Lung infection.

Signs:

- Fever
 - Cough
 - Confusion
-

COVID-19/Flu

Respiratory infections.

CNA Care

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- ✓ Observe breathing changes
 - ✓ Encourage deep breathing if ordered
 - ✓ Report oxygen issues
-

Emergency Signs

- ⚠ Blue lips
 - ⚠ Severe shortness of breath
 - ⚠ Chest pain
-

LESSON 6: DIGESTIVE SYSTEM

Function

Breaks down food for nutrients.

Common Conditions

GERD

Acid reflux.

Constipation

Common in elderly residents.

Signs:

- Pain
- No bowel movement

Diarrhea

Can lead to dehydration.

CNA Role

- ✓ Encourage fluids (if allowed)
 - ✓ Observe bowel changes
 - ✓ Report nausea/vomiting
-

LESSON 7: URINARY SYSTEM

Function

Removes waste through urine.

Common Diseases

Urinary Tract Infection (UTI)

Common in older adults.

Signs:

- Burning
 - Confusion
 - Fever
-

Kidney Disease

Impacts waste removal.

CNA Role

Report:

- ✓ Blood in urine
 - ✓ Burning
 - ✓ Strong odor
 - ✓ Confusion
-

LESSON 8: ENDOCRINE SYSTEM

Function

Controls hormones.

Common Disease: Diabetes

Body cannot regulate blood sugar.

Low Blood Sugar Signs

- ⚠ Sweating
 - ⚠ Shaking
 - ⚠ Confusion
-

High Blood Sugar Signs

- ⚠ Increased thirst
 - ⚠ Frequent urination
-

CNA Role

Report symptoms immediately.

Never give medication unless permitted.

LESSON 9: INTEGUMENTARY SYSTEM (SKIN)

Function

Protects body from injury and infection.

Common Conditions

Pressure Injuries (Pressure Ulcers)

Caused by prolonged pressure.

Stages range from mild redness to severe wounds.

Prevention

- ✓ Reposition residents
 - ✓ Keep skin clean and dry
 - ✓ Observe redness
-

LESSON 10: IMMUNE SYSTEM

Function

Protects body from infection.

Older adults often have weaker immune systems.

Signs of infection:

- ⚠ Fever
 - ⚠ Weakness
 - ⚠ Confusion
 - ⚠ Fatigue
-

LESSON 11: REPRODUCTIVE SYSTEM

Structure and function

- Female — ovaries, fallopian tubes, uterus, vagina, and breasts
- Male — testes, epididymis, vas deferens, prostate gland, and penis
- The system produces hormones, enables reproduction, and contributes to sexual identity and expression throughout life

Normal changes of aging

- Female — menopause; decreased estrogen; thinning and drying of vaginal tissue
- Male — decreased testosterone; enlargement of the prostate gland; slower sexual response
- Sexual desire and the need for intimacy and affection continue throughout life

Common conditions

- Benign prostatic hypertrophy — enlarged prostate causing urinary frequency, hesitancy, and retention
- Prostate cancer and breast cancer
- Vaginitis — inflammation of vaginal tissue
- Sexually transmitted infections, which occur in older adults as they do at any age

Resident rights and dignity

- Residents have the right to sexual expression and to privacy for consensual relationships
- Knock and wait for permission before entering; if you find residents in an intimate situation, leave quietly and provide privacy
- Never make jokes, express disapproval, or discuss a resident's sexual behavior with anyone other than the nurse
- Report any situation where a resident may not be able to consent, or where behavior is unwanted or directed at another resident or staff member

Observe and report

- Discharge, odor, bleeding, swelling, or lesions in the genital area
- Pain, burning, or difficulty with urination
- Lumps in the breast or testicle
- Any change in continence or urinary pattern

SKILLS PRACTICE ACTIVITY

Role-play reporting:

1. Shortness of breath
2. Sudden confusion
3. Chest pain
4. Skin redness

MODULE SUMMARY

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In this module, you learned:

- ✓ Major body systems
- ✓ Common diseases
- ✓ CNA observation responsibilities
- ✓ Emergency symptoms
- ✓ Disease-related care needs

Remember: CNAs do not diagnose disease, but your observations can save lives. Early reporting helps nurses intervene quickly and improve resident outcomes.

MODULE 8: BASIC NURSING SKILLS

Vital Signs, Height & Weight, Observation, Documentation & Reporting

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Estimated Completion Time: 4–5 Hours (Includes Extended Skills Lab Practice)

Textbook Reference: Nursing Assistant Care: The Basics, 6th Ed. (Hartman) | Ch 7, pp. 175–216

Federal & State Core Curriculum Alignment: Part II.A — Taking/Recording Vital Signs and Parameters | Part II.B — Measuring and Recording Height and Weight | Part II.C — Caring for the Residents' Environment | Part II.E — Recognizing and Reporting Pain

MODULE OVERVIEW

Basic nursing skills are essential responsibilities of the nurse aide. CNAs assist with observing residents, measuring vital signs, collecting information, reporting changes, and helping ensure resident safety and comfort.

In this module, students will learn and practice essential nursing assistant skills through **demonstration, return demonstration, simulation, and extended skills lab practice.**

This module includes **additional hands-on practice** to prepare students for **clinical rotations, competency testing, and state skills evaluation.**

Students will develop confidence in:

- Measuring vital signs
- Recording height and weight
- Pain observation
- Intake & output (I&O)
- Observation and reporting
- Documentation basics
- Communication with nurses
- Safe resident positioning
- Skill competency validation

Accuracy matters. What CNAs observe and report can save lives.

LEARNING OBJECTIVES

By the end of this module, learners will be able to:

1. Demonstrate proper measurement of vital signs.
 2. Accurately obtain height and weight.
 3. Recognize normal and abnormal findings.
 4. Observe and report changes in condition.
 5. Demonstrate proper documentation techniques.
 6. Explain intake and output monitoring.
 7. Use therapeutic communication when reporting concerns.
 8. Demonstrate competency through skills lab practice.
-

LESSON 1: INTRODUCTION TO BASIC NURSING SKILLS

What Are Basic Nursing Skills?

Basic nursing skills are routine care activities performed by nurse aides to support safe resident care.

These skills help monitor:

- Health status
 - Safety
 - Disease progression
 - Recovery
 - Changes in condition
-

CNA Responsibilities

CNAs:

- ✓ Observe residents carefully
- ✓ Report concerns immediately
- ✓ Follow care plans
- ✓ Record accurate information

CNAs do NOT:

- ✗ Diagnose conditions
- ✗ Interpret medical tests
- ✗ Change treatment plans

LESSON 2: VITAL SIGNS

What Are Vital Signs?

Vital signs are measurements used to monitor body function.

Vital signs include:

1. Temperature
 2. Pulse
 3. Respiration
 4. Blood Pressure
 5. Oxygen Saturation (Pulse Oximetry)
 6. Pain Observation
-

SECTION A: TEMPERATURE

Purpose

Measures body heat.

Normal Range

Approximately 97°F–99°F
(Facility policy may vary)

Temperature Routes

Oral

Mouth

Axillary

Under arm

Tympanic

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Ear

Temporal

Forehead

CNA Responsibilities

Report:

- ⚠ Fever
 - ⚠ Low temperature
 - ⚠ Sudden change
-

SKILLS LAB PRACTICE

Skill Practice #1: Taking Temperature

Students will:

- ✓ Gather equipment
- ✓ Identify resident
- ✓ Explain procedure
- ✓ Obtain temperature correctly
- ✓ Clean equipment
- ✓ Document findings

Required Practice:

Minimum 5 successful return demonstrations

SECTION B: PULSE

What Is Pulse?

Pulse measures heartbeats.

Common sites:

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Radial Pulse

Wrist (most common)

Carotid Pulse

Neck (emergency use)

Normal Adult Range

60–100 beats per minute

Observe:

- Fast pulse
 - Slow pulse
 - Irregular pulse
-

Reporting Concerns

Report immediately:

- ⚠ Very fast pulse
 - ⚠ Very slow pulse
 - ⚠ Irregular rhythm
-

SKILLS LAB PRACTICE

Skill Practice #2: Pulse Assessment

Students must:

- ✓ Locate pulse
- ✓ Count for 60 seconds
- ✓ Record accurately

Required Practice:

Minimum 10 pulse assessments

SECTION C: RESPIRATIONS

What Are Respirations?

Respirations are breaths taken per minute.

Observe:

- Rate
 - Rhythm
 - Effort
-

Normal Adult Range

12–20 breaths per minute

Report:

- ⚠ Shortness of breath
 - ⚠ Rapid breathing
 - ⚠ Labored breathing
 - ⚠ Wheezing
-

SKILLS LAB PRACTICE

Skill Practice #3: Respirations

Students must:

- ✓ Observe discreetly
- ✓ Count respirations correctly
- ✓ Report abnormalities

Required Practice:

Minimum 10 assessments

SECTION D: BLOOD PRESSURE

What Is Blood Pressure?

Measures pressure inside blood vessels.

Components

Systolic

Top number

Diastolic

Bottom number

CNA Responsibilities

Follow facility policy.

Report:

⚠ Very high BP

⚠ Very low BP

⚠ Dizziness symptoms

SKILLS LAB PRACTICE

Skill Practice #4: Manual Blood Pressure

Students will:

- ✓ Select cuff size
- ✓ Position arm correctly
- ✓ Use stethoscope properly
- ✓ Record reading

Required Practice:

Minimum 10 successful readings

SECTION E: OXYGEN SATURATION (SpO₂)

What Is Pulse Oximetry?

Measures oxygen in blood.

Normal ranges vary but often:

95%–100%

Report:

- ⚠ Low oxygen reading
 - ⚠ Shortness of breath
 - ⚠ Cyanosis (blue lips)
-

LESSON 3: HEIGHT & WEIGHT

Why Height and Weight Matter

Used to monitor:

- Nutrition
 - Hydration
 - Medication dosing
 - Weight gain/loss
-

Weight Guidelines

Weigh residents:

- Same time daily
 - Same clothing if possible
 - Same scale
-

Report:

⚠ Sudden weight loss

⚠ Sudden weight gain

SKILLS LAB PRACTICE

Skill Practice #5: Measuring Weight

Students must:

- ✓ Assist resident safely
- ✓ Zero scale
- ✓ Record findings

Required Practice:

Minimum 5 return demonstrations

LESSON 4: PAIN OBSERVATION

CNAs Do Not Diagnose Pain

But CNAs observe and report.

Signs include:

- Facial grimacing
- Crying
- Guarding body part

- Refusing movement
-

Example

Resident says:

“My chest hurts.”

Immediate action:

- ☒ Notify nurse immediately
-

LESSON 5: OBSERVATION & REPORTING

CNAs Are the Eyes and Ears of the Nurse

Report changes such as:

Physical Changes

- Fever
- Skin changes
- Falls
- Bleeding
- Weakness

Emotional Changes

- Sadness
- Confusion
- Agitation

Behavioral Changes

- Refusing meals
 - Sleeping changes
-

Important Rule

Report Immediately

Do NOT wait until end of shift.

LESSON 6: DOCUMENTATION

Why Documentation Matters

Documentation helps:

- Communicate care
 - Track resident progress
 - Improve safety
-

Documentation Rules

Document:

- ✓ Accurately
- ✓ Promptly
- ✓ Objectively

Never:

- ✗ Guess
 - ✗ Chart for others
 - ✗ Change information
-

Example

Poor documentation:

- ✗ “Resident looked bad.”

Better documentation:

- ✓ “Resident complained of dizziness and appeared weak.”

LESSON 7: INTAKE & OUTPUT (I&O)

Intake

Fluid entering body:

- Water
 - Juice
 - Soup
 - Ice cream
-

Output

Fluid leaving body:

- Urine
 - Stool
 - Vomit
-

Why Monitor I&O?

Helps assess:

- Hydration
 - Kidney function
 - Illness changes
-

LESSON 8: ADMISSION, TRANSFER & DISCHARGE

Moving into a facility, changing rooms, or going home are stressful events. How the nurse aide handles them shapes the resident's entire experience of the facility.

Admitting a resident

- Greet the resident by name and introduce yourself; a first impression is lasting
- Orient the resident to the room, call light, bathroom, bed controls, and dining and activity areas

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- Complete the personal property inventory in the resident's presence and have it signed
- Measure and record baseline height, weight, and vital signs
- Introduce the roommate and nearby staff; allow the resident and family time together
- Report the resident's condition, concerns, and questions to the nurse

Transferring a resident

- Explain the reason for the move and where the resident is going
- Move all personal belongings and verify against the property inventory
- Introduce the resident to staff and roommate in the new location
- Report completion of the transfer and the resident's response to the nurse

Discharging a resident

- A resident is discharged only on a physician's order
- Gather all belongings and verify against the property inventory
- Assist with dressing and packing; never rush the resident
- Accompany the resident to the exit and assist into the vehicle as directed
- The resident is the facility's responsibility until they leave the building

Remember: Never discuss a discharge date, a transfer, or a room change with a resident or family before the nurse has. Rumors about moving cause real distress.

LESSON 9: THE RESIDENT UNIT, SLEEP & BEDMAKING

Caring for the resident's unit

- Keep the call light within reach at all times — this is the single most important safety habit in the room
- Keep the over-bed table clean and uncluttered; it is used for meals and is never used for bedpans or soiled items
- Respect personal belongings; never move or discard anything without permission
- Maintain comfortable lighting, temperature, and noise level
- Report burned-out bulbs, broken equipment, spills, and malfunctioning beds

Sleep and rest

- Sleep supports healing, immune function, appetite, mood, and cognition
- Common disruptors include pain, noise, light, caffeine, napping, medications, needing to toilet, and anxiety
- Promote sleep by toileting before bedtime, offering a back rub, straightening linens, reducing noise and light, and honoring the resident's bedtime routine
- Report insomnia, daytime sleepiness, or a new change in sleep pattern to the nurse

Bedmaking

- A clean, dry, wrinkle-free bed prevents skin breakdown — wrinkles and crumbs cause pressure
- Wash hands and wear gloves if linen may be soiled
- Hold linen away from your uniform; never shake linen, which spreads microorganisms through the air
- Never place linen — clean or dirty — on the floor or on another resident's bed
- Place soiled linen in the designated container, not on the over-bed table or chair
- Unoccupied bed — made when the resident is out of bed
- Occupied bed — made with the resident in it, working one side at a time and keeping the resident covered and safe throughout

LESSON 10: CATHETERS, OXYGEN & IV THERAPY — THE NURSE AIDE'S ROLE

Nurse aides do not insert, remove, adjust, or discontinue any of these. The nurse aide provides supportive care and reports problems.

Urinary catheters

- Keep the drainage bag below the level of the bladder at all times, and never on the floor
- Keep tubing free of kinks and loops; never disconnect the tubing
- Provide perineal and catheter care daily and after each bowel movement
- Secure the tubing to the leg per facility policy to prevent pulling
- Empty the drainage bag and measure and record output as assigned
- Report cloudy, bloody, or foul-smelling urine, no urine output, leakage, pain, or a dislodged catheter immediately

Oxygen therapy

- A nurse aide never adjusts the oxygen flow rate — oxygen is a prescribed medication
- Check that tubing is not kinked and that the cannula or mask is positioned correctly
- Provide frequent mouth care; oxygen dries the mouth and nose
- Check the skin behind the ears, on the cheeks, and at the nostrils for pressure and irritation
- Observe fire safety — no smoking, no open flame, no electric razors, and no petroleum-based products near oxygen
- Report shortness of breath, bluish lips or nail beds, confusion, restlessness, or an empty tank immediately

IV therapy

- A nurse aide never adjusts the rate, disconnects the line, or takes a blood pressure on the arm with the IV
- Take care not to pull the tubing during transfers, dressing, and repositioning

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- Report swelling, redness, pain, bleeding, leaking, a dislodged needle, or an alarming pump to the nurse immediately

EXTENDED SKILLS LAB PRACTICE

Skill Stations

Station 1

Temperature & Pulse

Station 2

Respirations & Pulse Oximetry

Station 3

Blood Pressure

Station 4

Height & Weight

Station 5

Observation & Reporting

Station 6

Documentation Practice

MODULE SUMMARY

In this module, you learned:

- ✓ Vital signs
- ✓ Height and weight measurement
- ✓ Observation and reporting
- ✓ Documentation basics
- ✓ Intake & output
- ✓ Pain observation

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- ✓ Communication with nurses
- ✓ Extended skills lab practice

Remember: CNAs are the first to notice changes in residents. Accurate observations, vital signs, and timely reporting improve patient outcomes and save lives.

MODULE 9: PERSONAL CARE SKILLS

Bathing, Grooming, Dressing, Toileting & Hygiene Assistance

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Estimated Completion Time: 4–5 Hours

Textbook Reference: Nursing Assistant Care: The Basics, 6th Ed. (Hartman) | Ch 6, pp. 128–174

Skills Practice: Required In-Person Laboratory Training

Federal & State Core Curriculum Alignment: Part III.A — Bathing (incl. pressure ulcer observation) | Part III.B — Grooming, Including Mouth Care | Part III.C — Dressing | Part III.D — Toileting

MODULE TITLE

Personal Care Skills for the Nurse Aide

MODULE OVERVIEW

Personal care is one of the most important responsibilities of the nurse aide. Assisting residents with hygiene and activities of daily living (ADLs) promotes health, comfort, dignity, independence, and quality of life.

Many residents require assistance with bathing, dressing, grooming, oral care, toileting, and skin care due to illness, disability, weakness, or cognitive impairment.

Nurse aides must provide care that is:

- Safe
- Respectful
- Private
- Dignified
- Person-centered

This module provides foundational knowledge for personal care skills. **Hands-on competency training and skill validation will be completed in person during lab sessions at GA Healthcare Training & Consulting.**

LEARNING OBJECTIVES

By the end of this module, learners will be able to:

1. Explain the importance of personal hygiene.
 2. Identify personal care skills performed by CNAs.
 3. Demonstrate principles of dignity and privacy during care.
 4. Explain bathing and grooming procedures.
 5. Describe dressing assistance techniques.
 6. Explain toileting and incontinence care.
 7. Identify infection prevention during personal care.
 8. Demonstrate safety precautions during ADLs.
 9. Complete required in-person skills lab competencies.
-

LESSON 1: INTRODUCTION TO PERSONAL CARE

What Are Personal Care Skills?

Personal care skills help residents meet their daily hygiene and grooming needs.

Activities of Daily Living (ADLs) include:

- Bathing
 - Grooming
 - Oral care
 - Hair care
 - Nail care
 - Dressing
 - Toileting
 - Perineal care
 - Shaving
-

Why Personal Care Matters

Good hygiene helps:

- ✓ Prevent infection
- ✓ Improve comfort
- ✓ Promote dignity
- ✓ Prevent skin breakdown

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- ✓ Improve self-esteem
 - ✓ Support emotional well-being
-

CNA Responsibilities

CNAs should:

- ✓ Encourage independence
 - ✓ Promote privacy
 - ✓ Follow care plans
 - ✓ Observe skin changes
 - ✓ Report concerns
-

LESSON 2: BATHING ASSISTANCE

Purpose of Bathing

Bathing helps:

- Remove bacteria
 - Improve comfort
 - Prevent odors
 - Promote circulation
 - Observe skin condition
-

Types of Baths

Complete Bed Bath

For residents unable to leave bed.

Partial Bath

Focuses on face, hands, underarms, and perineal area.

Shower

For residents able to tolerate standing or seated bathing.

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Tub Bath

Performed when safe and ordered.

Bathing Principles

Always:

- ✓ Explain procedure
 - ✓ Gather supplies first
 - ✓ Provide privacy
 - ✓ Wear gloves when needed
 - ✓ Use warm water
 - ✓ Observe skin
-

Important Safety Tips

Never:

- ✗ Leave resident unattended in shower/tub
 - ✗ Use water that is too hot
 - ✗ Expose resident unnecessarily
-

SKILLS LAB (IN PERSON REQUIRED)

Practice Skill #1: Complete Bed Bath

Students must demonstrate:

- ✓ Hand hygiene
- ✓ Resident identification
- ✓ Privacy measures
- ✓ Proper bath sequence
- ✓ Safe positioning
- ✓ Skin observation

Required Practice:

Minimum 3 successful return demonstrations in lab

LESSON 3: GROOMING & PERSONAL HYGIENE

Grooming Includes

- Hair care
 - Oral care
 - Nail care
 - Shaving
 - Skin care
-

Oral Care

Good oral care prevents:

- Infection
 - Gum disease
 - Bad breath
 - Discomfort
-

Oral Care Steps

Always:

- ☒ Wear gloves
 - ☒ Brush teeth gently
 - ☒ Clean dentures properly
 - ☒ Offer mouth rinse
-

Denture Care

Handle carefully.

Never wrap dentures in tissue.

Always label denture containers.

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Hair Care

CNAs assist with:

- Brushing
 - Shampooing (per facility policy)
 - Styling assistance
-

Nail Care

Observe for:

- ⚠ Thick nails
- ⚠ Redness
- ⚠ Infection signs

Important: Follow facility policy for diabetic residents.

SKILLS LAB (IN PERSON REQUIRED)

Practice Skill #2: Grooming & Oral Care

Students must practice:

- ✓ Oral care
- ✓ Denture care
- ✓ Hair grooming
- ✓ Face care

Required Practice:

Minimum 5 successful demonstrations

LESSON 4: DRESSING & UNDRESSING

Why Dressing Assistance Matters

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Helps maintain:

- Independence
 - Comfort
 - Self-esteem
 - Safety
-

Dressing Principles

Allow resident to choose clothing whenever possible.

Encourage independence.

Maintain privacy.

Residents With Weakness

Dress Weak Side First

Undress Strong Side First

Example:

Stroke patient with weak left arm:

- ✅ Dress left arm first
-

Safety During Dressing

Avoid:

- ❌ Pulling forcefully
 - ❌ Rushing resident
-

LESSON 5: TOILETING & ELIMINATION

Importance of Toileting Assistance

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Proper toileting prevents:

- Skin breakdown
 - Embarrassment
 - Falls
 - Infection
-

Toileting Assistance Includes

- Bathroom assistance
 - Bedpan
 - Urinal
 - Commode use
 - Incontinence care
-

Toileting Safety

Always:

- ✓ Respond promptly to requests
 - ✓ Use gait belt if required
 - ✓ Lock wheelchair brakes
 - ✓ Provide privacy
-

Observe & Report

Report:

- ⚠ Blood in urine
 - ⚠ Constipation
 - ⚠ Diarrhea
 - ⚠ Painful urination
-

LESSON 6: PERINEAL CARE

What Is Perineal Care?

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Cleaning genital and anal areas to maintain hygiene and prevent infection.

Often needed:

- After toileting
 - During bathing
 - For incontinent residents
-

Infection Prevention

Always:

- ✓ Wear gloves
 - ✓ Clean front to back (especially females)
 - ✓ Use clean washcloth sections
-

Respect & Dignity

Explain procedure.

Maintain privacy.

Avoid embarrassment.

SKILLS LAB (IN PERSON REQUIRED)

Practice Skill #4: Perineal Care

Students must demonstrate:

- ✓ PPE use
- ✓ Proper cleansing technique
- ✓ Infection prevention
- ✓ Resident dignity

Required Practice:

Competency checkoff required

LESSON 7: INCONTINENCE CARE

What Is Incontinence?

Loss of bladder or bowel control.

CNA Responsibilities

Change residents promptly.

Keep skin:

- ☒ Clean
- ☒ Dry

Observe for:

- ⚠ Skin breakdown
 - ⚠ Odor
 - ⚠ Redness
-

Skin Protection

Frequent moisture may cause:

- Irritation
 - Pressure injuries
 - Infection
-

LESSON 8: SAFETY & OBSERVATION DURING PERSONAL CARE

CNAs should observe for:

Skin Changes

- Redness

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- Bruising
- Rashes
- Open wounds

Behavior Changes

- Confusion
- Pain
- Weakness

Hygiene Concerns

- Odor
 - Poor appetite
 - Neglect signs
-

Important Rule

Report concerns immediately.

EXTENDED IN-PERSON SKILLS LAB

Required Lab Stations

Station 1

Bed Bath

Station 2

Oral & Denture Care

Station 3

Hair Grooming

Station 4

Dressing Assistance

Station 5

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Toileting Assistance

Station 6

Perineal Care

Station 7

Incontinence Care

MODULE SUMMARY

In this module, you learned:

- ✓ Bathing assistance
- ✓ Grooming and oral care
- ✓ Dressing assistance
- ✓ Toileting support
- ✓ Perineal care
- ✓ Incontinence care
- ✓ Safety during personal care
- ✓ Observation and reporting

Remember: Personal care is more than helping with hygiene—it is about preserving dignity, independence, comfort, and quality of life. Hands-on mastery of these skills will be completed during your required in-person laboratory training.

MODULE 10: NUTRITION, HYDRATION & FEEDING TECHNIQUES

Nutrition Support, Safe Feeding & Fluid Assistance

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Estimated Completion Time: 4–5 Hours

Textbook Reference: Nursing Assistant Care: The Basics, 6th Ed. (Hartman) | Ch 8, pp. 217–234

Skills Practice: Required In-Person Laboratory Training

Federal & State Core Curriculum Alignment: Part III.E — Assisting with Eating and Hydration | Part III.F — Proper Feeding Techniques

MODULE TITLE

Nutrition, Hydration & Feeding Techniques for the Nurse Aide

MODULE OVERVIEW

Proper nutrition and hydration are essential to maintaining health, healing, strength, and quality of life. Many residents depend on nurse aides to assist with meals, encourage fluid intake, observe eating patterns, and recognize signs of nutritional problems.

Poor nutrition or dehydration may lead to:

- Weakness
- Falls
- Delayed healing
- Weight loss
- Confusion
- Infection
- Hospitalization

Nurse aides play an important role in helping residents eat safely and maintain adequate nutrition and hydration.

This module teaches nurse aides how to:

- Promote proper nutrition and hydration

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- Assist with feeding safely
- Identify special diets
- Prevent choking
- Recognize swallowing difficulties
- Monitor food and fluid intake
- Promote dignity during meals

Hands-on feeding techniques and meal assistance skills will be completed during required in-person laboratory training.

LEARNING OBJECTIVES

By the end of this module, learners will be able to:

1. Explain the importance of nutrition and hydration.
 2. Identify major nutrients and food groups.
 3. Describe common therapeutic diets.
 4. Demonstrate safe feeding techniques.
 5. Recognize signs of swallowing difficulty (dysphagia).
 6. Identify choking risks and emergency responses.
 7. Assist residents with meals while preserving dignity.
 8. Document food and fluid intake.
 9. Complete required feeding competencies in lab.
-

LESSON 1: INTRODUCTION TO NUTRITION & HYDRATION

Why Nutrition Matters

Food provides the body with:

- Energy
- Strength
- Healing
- Growth
- Disease prevention

Good nutrition helps residents:

- ✓ Maintain healthy weight
- ✓ Recover from illness

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- ✓ Heal wounds faster
 - ✓ Maintain strength
-

Why Hydration Matters

The body needs fluids to:

- Maintain body temperature
- Prevent dehydration
- Support organ function
- Prevent constipation
- Prevent confusion

Older adults are at increased risk for dehydration.

Signs of Dehydration

Watch for:

- ⚠ Dry mouth
 - ⚠ Confusion
 - ⚠ Dark urine
 - ⚠ Dizziness
 - ⚠ Weakness
 - ⚠ Decreased urination
-

LESSON 2: BASIC NUTRITION

Major Nutrients

Protein

Helps repair tissue.

Examples:

- Meat
- Eggs

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- Beans
 - Fish
-

Carbohydrates

Provide energy.

Examples:

- Bread
 - Rice
 - Pasta
-

Fats

Provide energy and protection.

Healthy fats are important.

Vitamins & Minerals

Support body function.

Examples:

- Fruits
 - Vegetables
-

Water

Essential for survival.

Balanced Diet

A healthy meal should include:

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- ✓ Protein
 - ✓ Fruits/vegetables
 - ✓ Grains
 - ✓ Fluids
-

LESSON 3: THERAPEUTIC DIETS

Some residents require special diets ordered by healthcare providers.

Common Diets

Regular Diet

No restrictions.

Soft Diet

Easy to chew.

Examples:

- Mashed potatoes
 - Soft vegetables
-

Mechanical Soft Diet

Food chopped or ground.

Pureed Diet

Smooth consistency.

For swallowing problems.

Low-Sodium Diet

Helps reduce fluid retention.

Often used for:

- Heart disease
 - High blood pressure
-

Diabetic Diet

Helps control blood sugar.

Thickened Liquids

Used for residents with swallowing difficulties.

Types include:

- Nectar thick
 - Honey thick
 - Pudding thick
-

Important Rule

Always follow the diet order exactly.

Never substitute food without approval.

LESSON 4: SAFE FEEDING TECHNIQUES

Before Feeding

Always:

- ☒ Wash hands
- ☒ Identify resident

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- ✓ Verify diet order
 - ✓ Position resident upright (90 degrees)
 - ✓ Ensure dentures/glasses if needed
-

During Feeding

Encourage independence first.

If assisting:

- ✓ Sit at eye level
 - ✓ Offer small bites
 - ✓ Feed slowly
 - ✓ Alternate food and fluids
 - ✓ Watch for swallowing
-

Never:

- ✗ Rush feeding
 - ✗ Feed lying flat
 - ✗ Leave resident unattended if unsafe
-

POSITIONING FOR FEEDING

Residents should sit:

Upright at 90 Degrees

This reduces aspiration risk.

Remain upright after meals if ordered.

SKILLS LAB (IN PERSON REQUIRED)

Practice Skill #1: Feeding Assistance

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Students must demonstrate:

- ✓ Hand hygiene
- ✓ Resident positioning
- ✓ Tray setup
- ✓ Safe feeding pace
- ✓ Therapeutic communication
- ✓ Dignity preservation

Required Practice:

Minimum 5 successful return demonstrations

LESSON 5: SWALLOWING DIFFICULTIES (DYSPHAGIA)

What Is Dysphagia?

Difficulty swallowing.

Common in residents with:

- Stroke
 - Dementia
 - Parkinson's disease
-

Warning Signs

- ⚠ Coughing while eating
 - ⚠ Pocketing food in cheeks
 - ⚠ Wet/gurgling voice
 - ⚠ Choking episodes
 - ⚠ Difficulty swallowing
-

CNA Responsibilities

Stop feeding if unsafe.

Report symptoms immediately.

Follow aspiration precautions.

LESSON 6: CHOKING PREVENTION & RESPONSE

Choking Risk Factors

- Fast eating
 - Large bites
 - Swallowing problems
 - Poor chewing
-

Choking Signs

- △ Cannot speak
 - △ Hands at throat
 - △ Difficulty breathing
 - △ Blue lips
-

CNA Action

Immediately:

- ✓ Get help
 - ✓ Follow facility emergency procedures
 - ✓ Alert trained personnel
-

LESSON 7: HYDRATION SUPPORT

Encouraging Fluids

CNAs should encourage fluids when allowed.

Offer:

- Water
- Juice

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- Soup
 - Ice chips (if allowed)
-

Residents at High Risk

- Elderly residents
 - Fever
 - Diarrhea
 - Poor appetite
-

Monitor Intake

Report:

- ⚠ Poor intake
 - ⚠ Refusal to eat/drink
 - ⚠ Sudden weight loss
-

LESSON 8: INTAKE & OUTPUT (I&O)

Intake Includes

- Water
 - Juice
 - Coffee
 - Ice cream
 - Soup
-

Output Includes

- Urine
 - Stool
 - Vomiting
-

Documentation

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Record:

- ✓ Amount eaten
 - ✓ Fluid intake
 - ✓ Meal refusals
-

Example

Resident ate:

- 75% breakfast
- 8 oz juice

Document accurately.

LESSON 9: OBSERVATION & REPORTING

Report immediately:

- ⚠ Refusal to eat
 - ⚠ Choking episodes
 - ⚠ Sudden weight loss
 - ⚠ Swallowing difficulty
 - ⚠ Vomiting
 - ⚠ Poor fluid intake
-

EXTENDED HANDS-ON LAB PRACTICE

Required Lab Stations

Station 1

Meal tray setup

Station 2

Resident positioning

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Station 3

Feeding assistance

Station 4

Dysphagia precautions

Station 5

Fluid intake monitoring

Station 6

Meal documentation

MODULE SUMMARY

In this module, you learned:

- ✓ Nutrition basics
- ✓ Hydration support
- ✓ Therapeutic diets
- ✓ Safe feeding techniques
- ✓ Dysphagia precautions
- ✓ Choking prevention
- ✓ Meal observation and reporting
- ✓ Hands-on feeding lab skills

Remember: Mealtime is more than nutrition—it is an opportunity to support health, dignity, safety, and emotional well-being. Safe feeding techniques can prevent aspiration and save lives.

MODULE 11: MENTAL HEALTH & PSYCHOSOCIAL WELL-BEING OF THE ELDERLY

Supporting Emotional Health, Cognitive Function & Quality of Life

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Estimated Completion Time: 3–4 Hours

Textbook Reference: Nursing Assistant Care: The Basics, 6th Ed. (Hartman) | Ch 3, pp. 63–82; Ch 5, pp. 115–127

Skills Practice: Communication & Behavioral Simulation in Lab

Federal & State Core Curriculum Alignment: Part IV — Mental Health and Social Service Needs | Part V — Care of Cognitively Impaired Residents

MODULE TITLE

Mental Health & Psychosocial Well-Being of the Elderly

MODULE OVERVIEW

Mental, emotional, and psychosocial well-being are important parts of healthy aging. Older adults may experience physical illness, grief, loneliness, memory changes, depression, anxiety, or cognitive decline.

Nurse aides play an important role in promoting emotional support, dignity, social interaction, and quality of life for elderly residents.

A resident's emotional needs are just as important as physical needs.

This module teaches nurse aides how to:

- Understand emotional and psychosocial needs of older adults
- Recognize normal aging versus mental illness
- Support residents with depression, anxiety, dementia, and confusion
- Promote emotional wellness and independence
- Communicate effectively with cognitively impaired residents
- Respond professionally to difficult behaviors
- Prevent social isolation and loneliness

Compassionate care improves emotional well-being and quality of life.

LEARNING OBJECTIVES

By the end of this module, learners will be able to:

1. Define psychosocial well-being.
 2. Identify emotional needs of older adults.
 3. Recognize signs of depression, anxiety, and loneliness.
 4. Differentiate dementia, delirium, and depression.
 5. Demonstrate communication strategies for cognitively impaired residents.
 6. Explain person-centered approaches to behavioral challenges.
 7. Promote independence and social engagement.
 8. Demonstrate therapeutic communication skills.
-

LESSON 1: UNDERSTANDING AGING & PSYCHOSOCIAL HEALTH

What Is Psychosocial Well-Being?

Psychosocial well-being includes:

- Emotional health
- Mental health
- Social relationships
- Sense of purpose
- Coping ability

Older adults may face life changes such as:

- Retirement
- Illness
- Loss of independence
- Death of spouse/friends
- Moving into long-term care

These changes may affect mental and emotional health.

Common Emotional Needs of Older Adults

Residents need:

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- ✓ Respect
 - ✓ Social connection
 - ✓ Independence
 - ✓ Meaningful activity
 - ✓ Emotional support
 - ✓ Privacy
 - ✓ Safety
-

CNA Role

CNAs can improve well-being by:

- ✓ Listening
 - ✓ Showing compassion
 - ✓ Encouraging participation
 - ✓ Respecting preferences
 - ✓ Building trust
-

LESSON 2: NORMAL AGING VS. MENTAL HEALTH CHANGES

Normal Aging Changes

Some changes may occur naturally:

- Slower movement
 - Mild forgetfulness
 - Reduced hearing/vision
 - Changes in sleep
-

Warning Signs That Are NOT Normal

Report:

- ⚠ Sudden confusion
- ⚠ Severe sadness
- ⚠ Withdrawal

⚠ Hallucinations

⚠ Major personality changes

Important Reminder

Confusion is **not always normal aging**.

Sudden confusion may indicate:

- Infection (UTI)
 - Medication reaction
 - Dehydration
 - Stroke
 - Illness
-

LESSON 3: DEPRESSION IN OLDER ADULTS

What Is Depression?

Depression is a medical condition affecting mood and daily functioning.

Older adults may experience depression because of:

- Illness
 - Loss
 - Loneliness
 - Reduced independence
-

Signs of Depression

⚠ Sadness

⚠ Withdrawal

⚠ Sleeping too much

⚠ Poor appetite

⚠ Hopelessness

⚠ Loss of interest

CNA Responsibilities

Observe and report.

Provide:

- ✓ Emotional support
- ✓ Encouragement
- ✓ Social interaction

Do NOT:

- ✗ Say “just be happy”
 - ✗ Ignore concerns
-

Therapeutic Communication Example

Resident:

“I don’t feel like life matters anymore.”

Poor response:

- ✗ “You’ll be fine.”

Better response:

- ✓ “I’m sorry you’re feeling this way. I’m here to listen.”

Then notify nurse.

LESSON 4: ANXIETY & STRESS

What Is Anxiety?

Anxiety is excessive worry or fear.

Common triggers:

- Illness

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- Fear of death
 - Changes in routine
 - Hospitalization
-

Signs of Anxiety

- ⚠ Restlessness
 - ⚠ Nervousness
 - ⚠ Increased worry
 - ⚠ Difficulty sleeping
-

CNA Support

- ✅ Stay calm
 - ✅ Reassure resident
 - ✅ Listen actively
 - ✅ Maintain routine
-

LESSON 5: DEMENTIA & ALZHEIMER'S DISEASE

What Is Dementia?

Dementia is loss of thinking ability severe enough to affect daily life.

Common Symptoms

- ⚠ Memory loss
 - ⚠ Wandering
 - ⚠ Confusion
 - ⚠ Agitation
 - ⚠ Forgetting people/places
-

Alzheimer's Disease

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Most common type of dementia.

Progressive disease affecting memory and behavior.

Communication Tips

DO:

- ☒ Speak calmly
- ☒ Use short sentences
- ☒ Maintain eye contact
- ☒ Give one instruction at a time
- ☒ Redirect gently

DO NOT:

- ☒ Argue
 - ☒ Correct harshly
 - ☒ Rush resident
-

Example

Resident says:

“I need to go pick up my children.”

Poor response:

- ☒ “Your children are adults.”

Better response:

- ☒ “Tell me about your children.”
-

LESSON 6: DELIRIUM VS DEMENTIA

Delirium

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Sudden confusion.

Usually caused by:

- Infection
- Illness
- Medication
- Dehydration

Medical emergency—report immediately.

Dementia

Slow, progressive memory decline.

Key Difference

Delirium = sudden

Dementia = gradual

LESSON 7: LONELINESS, GRIEF & SOCIAL ISOLATION

Why Elderly Residents May Feel Lonely

Residents may experience:

- Loss of family
 - Loss of spouse
 - Loss of independence
 - Isolation
-

Signs of Loneliness

⚠ Withdrawal

⚠ Sadness

⚠ Refusing activities

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CNA Role

Encourage:

- ✓ Conversation
 - ✓ Activities
 - ✓ Family interaction
 - ✓ Socialization
-

Grief in Older Adults

Residents may grieve:

- Death
- Illness
- Life changes

Allow residents to express feelings.

LESSON 8: DIFFICULT BEHAVIORS & PERSON-CENTERED CARE

Challenging Behaviors May Include

- Yelling
 - Aggression
 - Wandering
 - Refusing care
-

Possible Causes

- Pain
 - Fear
 - Confusion
 - Hunger
 - Anxiety
-

CNA Strategies

Stay calm.

Avoid arguing.

Redirect attention.

Maintain dignity.

Report concerns.

SKILLS LAB (IN PERSON REQUIRED)

Communication & Behavioral Simulation Lab

Students will practice:

- ✓ Therapeutic communication
 - ✓ Dementia communication
 - ✓ De-escalation techniques
 - ✓ Responding to depression signs
 - ✓ Managing anxiety
 - ✓ Redirection strategies
-

LESSON 9: GROWTH, DEVELOPMENT & DEVELOPMENTAL DISABILITIES

Stages of human growth and development

Understanding developmental stages helps the nurse aide respond to residents as whole people with a history, and helps explain behavior in residents of any age.

- Infancy through adolescence — rapid physical growth and the development of identity and independence
- Young adulthood — establishing relationships, career, and family
- Middle adulthood — productivity, contribution, and adjusting to physical change
- Late adulthood — adjusting to retirement, loss, and changing health; reviewing one's life
- Each stage carries developmental tasks; unmet tasks can surface as distress in later life

Developmental disabilities

A developmental disability is a permanent condition, present before age 22, that restricts physical or mental ability. Residents with developmental disabilities live in long-term care at every age.

- Examples include intellectual disability, cerebral palsy, Down syndrome, spina bifida, and autism spectrum disorder
- Ability varies enormously — never assume what a resident can or cannot do
- Speak to adults as adults; never use a childlike tone, nickname, or manner
- Promote independence and allow the time a task actually takes
- Use clear, simple language and consistent routines
- Involve the resident in choices about their own care

Remember: An adult with a developmental disability is an adult. Age-appropriate address and genuine choice are matters of dignity, not preference.

LESSON 10: THE NEED FOR ACTIVITY & CREATIVE THERAPIES

Why activity matters

- Physical — maintains strength, circulation, appetite, sleep, and bowel function; reduces pressure injury and contracture risk
- Psychological — reduces depression, anxiety, boredom, and agitation
- Social — preserves relationships, identity, and a sense of purpose
- Inactivity accelerates decline in every one of these areas

The nurse aide's role in activity

- Encourage and assist attendance at scheduled activities
- Ensure eyeglasses, hearing aids, dentures, and mobility devices are in place beforehand
- Escort residents to and from the activity area on time
- Report a resident who consistently declines activity or withdraws socially
- Never assume a resident is not interested — ask

Creative therapies in dementia care

For residents with Alzheimer's disease and other dementias, creative approaches reach past language loss and often reduce agitation more effectively than any verbal redirection.

- Music therapy — familiar music from the resident's youth can restore speech, memory, and mood
- Art therapy — drawing, painting, and coloring allow expression without words
- Reminiscence therapy — photographs, familiar objects, and life-story conversation
- Pet therapy — supervised animal visits that lower agitation and encourage interaction

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- Sensory activities — hand massage, textured objects, familiar scents, folding towels
- Exercise and movement — walking, chair exercise, and simple dancing

Remember: Sundowning and afternoon agitation often respond better to music, movement, or a familiar object than to any explanation. Try the activity before you try to reason.

MODULE SUMMARY

In this module, you learned:

- ✓ Psychosocial well-being
- ✓ Depression and anxiety
- ✓ Dementia and Alzheimer's disease
- ✓ Delirium vs dementia
- ✓ Therapeutic communication
- ✓ Loneliness and grief
- ✓ Difficult behaviors
- ✓ Person-centered emotional care

Remember: Caring for the elderly means caring for the whole person—mind, body, and spirit. Compassionate communication and emotional support can greatly improve quality of life for residents.

MODULE 12: MOBILITY, TRANSFERS & POSITIONING

Safe Resident Movement, Transfer Techniques & Body Mechanics

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Estimated Completion Time: 4–5 Hours

Textbook Reference: Nursing Assistant Care: The Basics, 6th Ed. (Hartman) | Ch 6, pp. 162–174; Ch 9, pp. 235–248

Skills Practice: Required In-Person Laboratory Training

Federal & State Core Curriculum Alignment: Part III.H — Transfers, Positioning and Turning (incl. pressure ulcer observation) | Part VI — Basic Restorative Services (Ambulation, ROM, Assistive Devices, Bowel/Bladder Training)

MODULE TITLE

Mobility, Transfers & Positioning for the Nurse Aide

MODULE OVERVIEW

Safe mobility, transfers, and positioning are essential nursing assistant skills. Many residents require assistance moving safely due to weakness, illness, surgery, disability, or aging-related changes.

Improper movement techniques may result in:

- Falls
- Resident injuries
- Skin breakdown
- Muscle weakness
- CNA back injuries
- Loss of independence

Nurse aides must use proper body mechanics, transfer techniques, and positioning methods to protect both residents and themselves.

This module teaches nurse aides how to:

- Assist residents with mobility safely
- Apply proper body mechanics

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- Transfer residents using approved techniques
- Use transfer equipment safely
- Reposition residents to prevent complications
- Prevent falls and injuries
- Promote resident independence

Hands-on competency practice will be completed during required in-person skills laboratory training.

LEARNING OBJECTIVES

By the end of this module, learners will be able to:

1. Explain the importance of safe mobility.
 2. Demonstrate proper body mechanics.
 3. Identify safe transfer techniques.
 4. Demonstrate wheelchair and gait belt safety.
 5. Describe proper resident positioning.
 6. Explain fall prevention during movement.
 7. Promote resident independence while maintaining safety.
 8. Complete required mobility competencies in lab.
-

LESSON 1: INTRODUCTION TO MOBILITY

What Is Mobility?

Mobility is a person's ability to move safely and independently.

Mobility may include:

- Walking
 - Standing
 - Turning
 - Sitting
 - Repositioning
 - Transferring
-

Why Mobility Matters

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Movement helps:

- ✓ Maintain strength
 - ✓ Improve circulation
 - ✓ Prevent stiffness
 - ✓ Prevent pressure injuries
 - ✓ Improve breathing
 - ✓ Promote independence
-

Risks of Immobility

Lack of movement may lead to:

- ⚠ Muscle weakness
 - ⚠ Pressure injuries
 - ⚠ Falls
 - ⚠ Pneumonia
 - ⚠ Blood clots
 - ⚠ Constipation
-

CNA Responsibilities

CNAs should:

- ✓ Encourage movement when safe
 - ✓ Follow care plans
 - ✓ Use assistive devices properly
 - ✓ Report mobility changes
-

LESSON 2: BODY MECHANICS

Why Body Mechanics Matter

Poor lifting techniques may cause:

- Back injuries
- Muscle strain

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- Falls

Good body mechanics protect:

- ☒ Resident safety
 - ☒ CNA safety
-

Proper Body Mechanics

Stand With Wide Base of Support

Feet shoulder-width apart.

Bend Knees

Lift using leg muscles.

Keep Back Straight

Avoid bending at waist.

Hold Objects Close

Reduces strain.

Avoid Twisting

Turn entire body.

Ask for Help

Never lift alone if unsafe.

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Important Rule

If a transfer feels unsafe, stop and get assistance.

SKILLS LAB (IN PERSON REQUIRED)

Practice Skill #1: Proper Body Mechanics

Students must demonstrate:

- ✓ Safe stance
- ✓ Proper lifting technique
- ✓ Turning safely
- ✓ Use of leg muscles

Required Practice:

Minimum 5 successful demonstrations

LESSON 3: ASSISTIVE DEVICES

Residents may use:

Walkers

Canes

Wheelchairs

Mechanical Lifts

Transfer Belts (Gait Belts)

Always follow:

- ✓ Care plan
- ✓ Facility policy

Wheelchair Safety

Always:

- ✓ Lock brakes
- ✓ Remove footrests when needed
- ✓ Position wheelchair properly

Never:

- ✗ Move resident without locking wheels
-

LESSON 4: TRANSFER TECHNIQUES

What Is a Transfer?

Moving a resident safely from one place to another.

Examples:

- Bed to wheelchair
 - Wheelchair to toilet
 - Bed to chair
-

Before Every Transfer

Always:

- ✓ Wash hands
 - ✓ Identify resident
 - ✓ Explain procedure
 - ✓ Lock wheels
 - ✓ Use proper footwear
 - ✓ Gather assistance if needed
-

One-Person Transfer

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Used when resident can assist.

Two-Person Transfer

Used when resident requires more help.

Mechanical Lift Transfer

Used for residents unable to bear weight.

Only trained staff should assist.

GAIT BELT SAFETY

What Is a Gait Belt?

A safety belt used to assist residents during transfers and walking.

Proper Use

Place securely around waist.

Do NOT:

✗ Place over wounds or tubes

SKILLS LAB (IN PERSON REQUIRED)

Practice Skill #2: Bed-to-Wheelchair Transfer

Students must demonstrate:

- ✓ Resident communication
- ✓ Brake safety

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- ✓ Gait belt use
- ✓ Proper positioning
- ✓ Safe transfer

Required Practice:

Minimum 10 successful return demonstrations

LESSON 5: AMBULATION (WALKING ASSISTANCE)

Why Ambulation Matters

Walking helps:

- ✓ Maintain strength
 - ✓ Improve circulation
 - ✓ Prevent complications
-

Safe Ambulation Steps

Always:

- ✓ Apply gait belt
 - ✓ Walk beside resident
 - ✓ Wear non-slip shoes
 - ✓ Watch for fatigue
-

Weak Side Rule

Walk slightly behind and beside resident on **weak side**.

LESSON 6: POSITIONING RESIDENTS

Why Positioning Matters

Proper positioning helps prevent:

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- Pressure injuries
 - Contractures
 - Pain
 - Poor circulation
-

Common Positions

Supine Position

Flat on back.

Side-Lying Position

Resident lies on side.

Fowler's Position

Head elevated.

Common for:

- Eating
 - Breathing problems
-

Sims' Position

Semi-side position.

Repositioning Schedule

Residents may require turning:

Every 2 Hours

(or facility policy)

PRESSURE INJURY PREVENTION

Observe:

- ⚠ Redness
- ⚠ Skin breakdown
- ⚠ Pain

Always:

- ✅ Use pillows for support
 - ✅ Keep sheets wrinkle-free
 - ✅ Reposition regularly
-

SKILLS LAB (IN PERSON REQUIRED)

Practice Skill #3: Repositioning Resident in Bed

Students will demonstrate:

- ✓ Turning resident safely
- ✓ Pillow placement
- ✓ Pressure relief techniques

Required Practice:

Minimum 5 return demonstrations

LESSON 7: RANGE OF MOTION (ROM)

What Is ROM?

Exercises that keep joints flexible.

Types

Active ROM

Resident moves independently.

Passive ROM

CNA assists movement.

Safety Rules

Never:

- ✗ Force movement
- ✗ Cause pain

Stop if resident reports discomfort.

LESSON 8: FALL PREVENTION DURING MOBILITY

Fall Risk Factors

Residents may fall due to:

- Weakness
 - Confusion
 - Poor balance
 - Medications
-

Prevention Strategies

Always:

- ✓ Keep call light near
 - ✓ Use proper footwear
 - ✓ Clear clutter
 - ✓ Assist when needed
-

If a Resident Begins to Fall

Do NOT try to stop fall abruptly.

Instead:

- ✓ Ease resident safely to floor if possible
- ✓ Protect head
- ✓ Call for help

Never:

- ✗ Lift resident alone after fall
-

LESSON 9: PROSTHETIC & ORTHOTIC DEVICES

A prosthesis replaces a missing body part; an orthosis (brace or splint) supports, aligns, or protects a body part. Both are costly, individually fitted, and essential to the resident's function and self-image.

Common devices

- Artificial limbs — leg and arm prostheses
- Artificial eye and breast prosthesis
- Braces and splints — back, knee, ankle-foot orthosis, wrist splint
- Hearing aids, eyeglasses, and dentures are also prosthetic devices

Care guidelines

- Apply and remove the device exactly as directed in the care plan and only after being trained
- Check the skin under and around the device every time it is applied or removed
- Keep the skin and the device clean and completely dry before application
- Never force a device that does not fit; stop and report it
- Store the device safely in the labeled place — never on the floor, in a bed, or on a meal tray
- Handle every device carefully; replacement is expensive and takes time

Observe and report

- Redness, swelling, blistering, open areas, or skin breakdown under or around the device
- Pain, numbness, tingling, or coolness in the limb
- A device that has become loose, tight, cracked, worn, or malfunctioning

- A resident who refuses to wear a prescribed device

LESSON 10: BLADDER & BOWEL RETRAINING

Incontinence is not a normal part of aging. Retraining programs restore continence for many residents, and success depends almost entirely on the consistency of the nurse aide.

Bladder retraining

- Follow the scheduled toileting times in the care plan exactly
- Offer toileting on waking, before and after meals, at bedtime, and during the night as scheduled
- Offer adequate fluids — restricting fluids worsens incontinence and causes dehydration and urinary tract infection
- Provide privacy and enough uninterrupted time; rushing defeats the program
- Answer call lights for toileting immediately
- Record every void, every episode of incontinence, and the amount when measured

Bowel retraining

- Toilet at the same time each day, taking advantage of the urge after a meal
- Position the resident upright with feet supported
- Encourage fiber, fluids, and activity as permitted by the care plan
- Record every bowel movement, its consistency, and any difficulty

Professional response to incontinence

- Never scold, sigh, rush, or show disgust — incontinence is already humiliating
- Change the resident promptly, provide perineal care, and check the skin every time
- Never refer to incontinence products as diapers
- Report skin redness or breakdown, pain or burning on urination, cloudy or foul-smelling urine, constipation, diarrhea, or any change in the usual pattern

Remember: A retraining program only works if every aide on every shift follows the schedule. One shift that skips it undoes weeks of progress.

EXTENDED SKILLS LAB PRACTICE

Required Skill Stations

Station 1

Body mechanics

Station 2

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Wheelchair transfer

Station 3

Gait belt application

Station 4

Ambulation assistance

Station 5

Resident positioning

Station 6

ROM exercises

MODULE SUMMARY

In this module, you learned:

- ✓ Safe mobility principles
- ✓ Body mechanics
- ✓ Transfer techniques
- ✓ Gait belt use
- ✓ Ambulation assistance
- ✓ Resident positioning
- ✓ Range of motion exercises
- ✓ Fall prevention

Remember: Safe movement protects both the resident and the caregiver. Proper transfer and positioning techniques help prevent injuries, preserve dignity, and improve quality of life. Hands-on mastery will be completed during your required skills lab training.

MODULE 13: SKIN & WOUND CARE

Skin Integrity, Pressure Injury Prevention & Basic Wound Observation

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Estimated Completion Time: 3–4 Hours

Textbook Reference: Nursing Assistant Care: The Basics, 6th Ed. (Hartman) | Ch 4, p. 83; Ch 6, pp. 129–132; Ch 7, p. 214

Skills Practice: Required In-Person Laboratory Training

Federal & State Core Curriculum Alignment: Part III.G — Skin Care; Observation for Pressure Ulcers and Skin Tears

MODULE TITLE

Skin & Wound Care for the Nurse Aide

MODULE OVERVIEW

The skin is the body’s largest organ and serves as a protective barrier against injury, infection, and illness. Older adults are at higher risk for skin breakdown due to aging, immobility, poor nutrition, incontinence, chronic disease, and decreased circulation.

Nurse aides play an important role in:

- Protecting skin integrity
- Preventing pressure injuries
- Observing wounds and skin changes
- Promoting hygiene and nutrition
- Reporting concerns promptly

CNAs do not perform sterile wound care unless permitted by policy and training. Their role is observation, prevention, hygiene assistance, and reporting.

This module teaches nurse aides how to:

- Understand skin function
- Prevent skin breakdown
- Recognize pressure injuries
- Observe wounds and report concerns
- Assist with skin hygiene

- Promote healing through prevention strategies

Hands-on skin assessment observation and prevention skills will be practiced during in-person skills lab training.

LEARNING OBJECTIVES

By the end of this module, learners will be able to:

1. Explain the function of the skin.
 2. Identify factors affecting skin health.
 3. Describe stages of pressure injuries.
 4. Demonstrate skin breakdown prevention.
 5. Identify wound warning signs.
 6. Explain CNA responsibilities in wound observation.
 7. Demonstrate safe repositioning for skin protection.
 8. Report skin changes appropriately.
-

LESSON 1: INTRODUCTION TO SKIN CARE

What Is the Skin?

The skin is the body's largest organ.

Functions include:

Protection

Protects against germs and injury.

Temperature Regulation

Helps maintain body temperature.

Sensation

Allows feeling of touch, pain, heat, and cold.

Prevention of Fluid Loss

Keeps body hydrated.

Why Skin Care Matters in Elderly Residents

Older adults often have:

- Thin skin
- Fragile tissue
- Reduced circulation
- Limited mobility

These factors increase risk for:

- ⚠ Skin tears
 - ⚠ Bruising
 - ⚠ Pressure injuries
 - ⚠ Infection
-

LESSON 2: FACTORS THAT AFFECT SKIN HEALTH

Risk Factors for Skin Breakdown

Immobility

Staying in one position too long.

Poor Nutrition

Lack of protein and hydration.

Incontinence

Moisture damages skin.

Chronic Disease

Diabetes and poor circulation slow healing.

Friction & Shearing

Skin rubbing against surfaces.

Common High-Risk Areas

Pressure injuries often develop on bony areas:

- Heels
 - Hips
 - Elbows
 - Tailbone (coccyx)
 - Ankles
 - Shoulders
-

LESSON 3: PRESSURE INJURIES (PRESSURE ULCERS)

What Is a Pressure Injury?

Damage to skin caused by prolonged pressure.

Also called:

- Pressure ulcers
 - Bed sores
 - Decubitus ulcers
-

Causes

Pressure decreases blood flow to skin.

Residents at high risk:

- Bed-bound
 - Wheelchair users
 - Poor nutrition
 - Limited movement
-

STAGES OF PRESSURE INJURIES

Stage 1

Signs:

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⚠ Redness that does not go away

⚠ Skin intact

Stage 2

Signs:

⚠ Open blister or shallow wound

Stage 3

Signs:

⚠ Deeper wound into tissue

Stage 4

Signs:

⚠ Severe tissue damage

Important CNA Rule

CNAs do not stage wounds.

CNAs:

- ✓ Observe
 - ✓ Report changes immediately
-

LESSON 4: PRESSURE INJURY PREVENTION

Prevention Is Key

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CNAs play a major role in prevention.

Repositioning

Residents may need repositioning:

Every 2 Hours

(or care plan/facility policy)

Pressure Relief Techniques

Use:

- ☒ Pillows for support
 - ☒ Heel protectors
 - ☒ Pressure-relieving mattresses
-

Skin Hygiene

Keep skin:

- ☒ Clean
- ☒ Dry

Especially after:

- Sweating
 - Incontinence
 - Bathing
-

Nutrition & Hydration

Healthy skin needs:

- Protein
- Fluids

- Vitamins
-

SKILLS LAB (IN PERSON REQUIRED)

Practice Skill #1: Repositioning for Skin Protection

Students will demonstrate:

- ✓ Turning resident safely
- ✓ Pillow placement
- ✓ Heel offloading
- ✓ Pressure reduction

Required Practice:

Minimum 5 successful demonstrations

LESSON 5: WOUND OBSERVATION

CNA Role in Wound Care

CNAs:

- ✓ Observe
- ✓ Report
- ✓ Follow care plan

CNAs do NOT:

- ✗ Change dressings unless trained and allowed
 - ✗ Diagnose wounds
-

Report Immediately

- ⚠ Redness
- ⚠ Drainage
- ⚠ Bad odor

- ⚠ Swelling
 - ⚠ Bleeding
 - ⚠ Pain increase
 - ⚠ Skin tears
-

Signs of Infection

Watch for:

- ⚠ Redness
 - ⚠ Warmth
 - ⚠ Swelling
 - ⚠ Fever
 - ⚠ Drainage
-

LESSON 6: SKIN TEARS & BRUISING

Skin Tears

Older adults have fragile skin.

Causes:

- Transfers
 - Friction
 - Bumping into objects
-

Prevention

Always:

- ✅ Move residents carefully
 - ✅ Avoid dragging skin
 - ✅ Use proper transfer techniques
-

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Bruising

Observe for:

- ⚠ Large bruises
- ⚠ Unexplained injuries

May indicate:

- Trauma
 - Medication effects
 - Abuse concerns
-

LESSON 7: INCONTINENCE & SKIN CARE

Moisture Damage

Urine and stool may damage skin.

CNA Responsibilities

Change residents promptly.

Keep skin:

- ☒ Clean
- ☒ Dry

Use:

- ☒ Barrier creams (per policy)

Observe for:

- ⚠ Redness
 - ⚠ Rash
 - ⚠ Skin breakdown
-

LESSON 8: DOCUMENTATION & REPORTING

Accurate Reporting Matters

Report skin concerns immediately.

Example:

Poor report:

✗ “Skin looks bad.”

Better report:

✓ “Resident has redness on coccyx approximately quarter-sized.”

Documentation Principles

Document:

- ✓ Accurate observations
- ✓ Objective findings
- ✓ Timely reporting

Never guess.

EXTENDED SKILLS LAB PRACTICE

Required Lab Stations

Station 1

Skin observation

Station 2

Repositioning techniques

Station 3

Pressure relief positioning

Station 4

Incontinence skin care

Station 5

Reporting/documentation

MODULE SUMMARY

In this module, you learned:

- ✓ Skin function
- ✓ Pressure injury prevention
- ✓ Wound observation
- ✓ Infection signs
- ✓ Skin tear prevention
- ✓ Incontinence skin care
- ✓ Reporting and documentation

Remember: Healthy skin protects the body. CNAs play a critical role in preventing pressure injuries and identifying early signs of skin breakdown. Early reporting can prevent serious complications and improve resident outcomes.

MODULE 14: END-OF-LIFE CARE

Comfort Care, Hospice Support, Grief & Dignified Care at the End of Life

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Estimated Completion Time: 3–4 Hours

Skills Practice: Communication & Comfort Care Simulation in Lab

Textbook Reference: Nursing Assistant Care: The Basics, 6th Ed. (Hartman) | Ch 3, pp. 75–82

Federal & State Core Curriculum Alignment: Part II.F — Caring for a Resident When Death is Imminent and Post-Mortem Care

MODULE TITLE

End-of-Life Care for the Nurse Aide

MODULE OVERVIEW

End-of-life care is compassionate care provided to residents who are nearing death. Nurse aides play an important role in promoting comfort, dignity, emotional support, and quality of life during this sensitive time.

Residents at the end of life may experience:

- Physical decline
- Pain
- Emotional distress
- Anxiety
- Spiritual concerns
- Fear of dying

Families may also experience grief, stress, and emotional hardship.

Nurse aides help residents and families by providing:

- Comfort
- Compassion
- Respect
- Emotional support
- Personal care
- Dignity

The goal of end-of-life care is comfort and quality of life—not cure.

LEARNING OBJECTIVES

By the end of this module, learners will be able to:

1. Define end-of-life care and hospice care.
2. Recognize signs of approaching death.
3. Explain CNA responsibilities during end-of-life care.
4. Demonstrate compassionate communication.
5. Provide comfort-focused personal care.

6. Support families during grief.
 7. Respect cultural and spiritual preferences.
 8. Describe postmortem care basics.
-

LESSON 1: UNDERSTANDING END-OF-LIFE CARE

What Is End-of-Life Care?

End-of-life care focuses on:

- Comfort
- Pain management
- Dignity
- Emotional support
- Quality of life

The goal is **comfort rather than cure**.

What Is Hospice Care?

Hospice care provides support for residents with terminal illness.

Hospice focuses on:

- ✓ Comfort care
 - ✓ Pain relief
 - ✓ Emotional support
 - ✓ Family support
-

Palliative Care vs Hospice

Palliative Care

Comfort-focused care at any stage of illness.

Hospice Care

Care for residents nearing end of life.

LESSON 2: SIGNS OF APPROACHING DEATH

Residents nearing death may experience physical changes.

Common Signs

Decreased Appetite & Thirst

Resident may eat or drink less.

Increased Sleep

More sleeping or unresponsiveness.

Changes in Breathing

May include:

- Slow breathing
 - Irregular breathing
 - Periods of no breathing
-

Cool or Mottled Skin

Hands and feet may become cool.

Confusion or Withdrawal

Resident may become less responsive.

Important Reminder

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These changes are often natural.

CNAs should:

- ✓ Stay calm
 - ✓ Provide comfort
 - ✓ Report changes to nurse
-

LESSON 3: COMFORT MEASURES

CNA Role in Comfort Care

CNAs help reduce discomfort by:

Repositioning

Prevent pain and pressure injuries.

Mouth Care

Dry mouth is common.

Provide oral care frequently.

Skin Care

Keep resident clean and dry.

Quiet Environment

Reduce noise and stress.

Gentle Touch

Offer reassurance if welcomed.

Pain Observation

CNAs should observe for:

- ⚠ Facial grimacing
- ⚠ Restlessness
- ⚠ Moaning
- ⚠ Changes in breathing

Report concerns promptly.

LESSON 4: EMOTIONAL & PSYCHOSOCIAL SUPPORT

Residents may feel:

- Fear
 - Sadness
 - Anxiety
 - Loneliness
-

Therapeutic Communication

Sometimes listening is the best care.

Example:

Resident says:

“I’m scared.”

Poor response:

✗ “Don’t think like that.”

Better response:

✓ “I’m here with you. Would you like to talk?”

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Important Rule

Allow residents to express feelings.

Do NOT:

- ✗ Argue
 - ✗ Minimize emotions
 - ✗ Avoid difficult conversations
-

LESSON 5: FAMILY SUPPORT & GRIEF

Families may experience:

- Sadness
 - Stress
 - Anger
 - Fear
 - Exhaustion
-

CNA Role With Families

Provide:

- ✓ Respect
- ✓ Compassion
- ✓ Quiet support

Do NOT:

- ✗ Give medical advice
 - ✗ Share opinions outside role
-

Professional Response Example

Family asks:

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“How much longer?”

Better response:

- ✓ “I know this is difficult. The nurse may be able to answer your questions.”
-

LESSON 6: CULTURAL & SPIRITUAL CONSIDERATIONS

Residents may have:

- Religious beliefs
 - Spiritual traditions
 - Cultural practices
-

Respect Preferences

Examples:

- ✓ Prayer requests
 - ✓ Religious objects
 - ✓ Family traditions
-

Important Rule

Never judge beliefs.

Provide respectful care.

LESSON 7: DIGNITY AT THE END OF LIFE

Residents deserve dignity until the very end.

Always:

- ✓ Speak respectfully
- ✓ Explain care

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- ✓ Protect privacy
 - ✓ Maintain grooming
 - ✓ Address resident by preferred name
-

Important Reminder

Even if a resident appears unconscious:

Continue speaking respectfully.

Example:

“Mr. Jones, I’m going to help reposition you now.”

LESSON 8: POSTMORTEM CARE

What Is Postmortem Care?

Care provided after death.

CNA Responsibilities

May include (per policy):

- ✓ Respectful handling of body
 - ✓ Personal hygiene care
 - ✓ Privacy measures
 - ✓ Supporting family presence
-

Important Rule

Always follow:

- Facility policy
- Nurse instructions
- Cultural considerations

SKILLS LAB (IN PERSON REQUIRED)

Communication & Comfort Care Simulation Lab

Students will practice:

- ✓ Comfort positioning
 - ✓ Mouth care
 - ✓ Therapeutic communication
 - ✓ Family interaction
 - ✓ Emotional support
-

MODULE SUMMARY

In this module, you learned:

- ✓ End-of-life care principles
- ✓ Hospice and palliative care
- ✓ Signs of approaching death
- ✓ Comfort care measures
- ✓ Emotional and family support
- ✓ Cultural sensitivity
- ✓ Dignity preservation
- ✓ Postmortem care basics

Remember: End-of-life care is about compassion, dignity, and comfort. The kindness and respect you show during a resident's final days may leave a lasting impact on both the resident and their family.

MODULE 15: ETHICS, LEGAL ISSUES & REGULATORY OVERVIEW

Professional Responsibility, Resident Protection & CNA Legal Standards

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Estimated Completion Time: 3–4 Hours

Textbook Reference: Nursing Assistant Care: The Basics, 6th Ed. (Hartman) | Ch 1, pp. 8–20; Ch 10, pp. 249–257

Final Module – CNA Program Completion

Federal & State Core Curriculum Alignment: Part I.E — Legal/Ethical Behavior and Scope of Practice | Part VII — Residents' Rights (Privacy, Confidentiality, Grievances, Personal Choices, Restraints)

MODULE TITLE

Ethics, Legal Issues & Regulatory Overview for the Nurse Aide

MODULE OVERVIEW

Nurse aides are responsible for providing safe, ethical, legal, and professional care to residents. CNAs must understand laws, regulations, resident protections, professional boundaries, confidentiality, and ethical decision-making.

Failure to follow legal and ethical standards may result in:

- Resident harm
- Loss of certification
- Disciplinary action
- Termination
- Civil or criminal consequences

This final module prepares students to understand their professional responsibilities and safely transition into clinical practice.

This module teaches nurse aides how to:

- Apply ethical principles in caregiving
- Understand resident rights and legal protections
- Maintain confidentiality and privacy

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- Practice within CNA scope of practice
- Recognize negligence and liability
- Understand documentation responsibilities
- Follow state and federal regulations
- Prepare for safe professional practice

Professionalism, ethics, and accountability are essential to safe nursing assistant practice.

LEARNING OBJECTIVES

By the end of this module, learners will be able to:

1. Define ethics and legal responsibility.
 2. Identify professional standards for CNAs.
 3. Explain resident legal rights and protections.
 4. Describe scope of practice and delegation.
 5. Recognize negligence, malpractice, and liability risks.
 6. Explain confidentiality and HIPAA.
 7. Identify regulatory agencies affecting CNA practice.
 8. Demonstrate ethical decision-making.
 9. Apply professional accountability in resident care.
-

LESSON 1: INTRODUCTION TO ETHICS

What Are Ethics?

Ethics are moral principles that guide behavior and decision-making.

Healthcare ethics help professionals determine:

- What is right
 - What is fair
 - What is safe
 - What protects dignity
-

Ethical Principles in Healthcare

Respect for Dignity

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Treat every resident respectfully.

Autonomy

Residents have the right to make decisions.

Beneficence

Doing good for residents.

Nonmaleficence

“Do no harm.”

Justice

Treat everyone fairly.

CNA Ethical Responsibilities

CNAs should:

- ☒ Be honest
 - ☒ Respect residents
 - ☒ Protect privacy
 - ☒ Maintain professionalism
-

LESSON 2: LEGAL RESPONSIBILITIES OF CNAs

What Are Legal Responsibilities?

Legal responsibilities are duties required by law.

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CNAs must:

- ✓ Follow facility policy
 - ✓ Follow state regulations
 - ✓ Work within scope of practice
 - ✓ Protect resident safety
-

Scope of Practice

CNAs may:

- ✓ Assist with ADLs
- ✓ Observe and report
- ✓ Obtain vital signs

CNAs may NOT:

- ✗ Diagnose illness
 - ✗ Prescribe medication
 - ✗ Perform procedures outside training
-

Delegation

Nurses may delegate tasks.

CNAs should:

- ✓ Ask questions if unsure
 - ✓ Refuse unsafe tasks appropriately
-

LESSON 3: NEGLIGENCE, ABUSE & LIABILITY

What Is Negligence?

Failure to provide proper care.

Examples:

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- ✗ Ignoring call lights
 - ✗ Leaving bed alarm off
 - ✗ Not reporting symptoms
-

Consequences of Negligence

May result in:

- Resident injury
 - Lawsuits
 - Job loss
 - Certification issues
-

Examples of Negligence

Resident falls because wheelchair brakes were not locked.

This may be:

- ✓ Negligence
-

Abuse & Legal Reporting

CNAs are **mandatory reporters**.

Must report:

- ✓ Abuse
 - ✓ Neglect
 - ✓ Misappropriation
-

LESSON 4: CONFIDENTIALITY & HIPAA

What Is Confidentiality?

Resident information is private.

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HIPAA Basics

The **Health Insurance Portability and Accountability Act (HIPAA)** protects resident information.

CNAs should:

- ☒ Keep information private
- ☒ Share only with healthcare team

Never:

- ☒ Discuss residents publicly
 - ☒ Share on social media
 - ☒ Access records without reason
-

Example

Incorrect:

- ☒ Talking about resident diagnosis in elevator.

Correct:

- ☒ Private communication with nurse.
-

LESSON 5: PROFESSIONALISM & PROFESSIONAL BOUNDARIES

Professionalism Includes

- Respect
 - Reliability
 - Accountability
 - Compassion
 - Teamwork
-

Professional Boundaries

Avoid:

-  Accepting expensive gifts
-  Sharing personal problems excessively
-  Borrowing money

Maintain therapeutic relationships.

Attendance & Dependability

Professional CNAs:

-  Arrive on time
 -  Communicate concerns
 -  Follow policies
-

LESSON 6: DOCUMENTATION & LEGAL RECORDS

Why Documentation Matters

Documentation is a legal record.

Documentation Principles

Document:

-  Accurate facts
-  Timely observations
-  Objective findings

Never:

-  Guess
-  Chart false information
-  Document for someone else

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Example

Poor documentation:

✗ “Resident looked terrible.”

Better documentation:

✓ “Resident complained of dizziness and appeared pale.”

LESSON 7: REGULATORY AGENCIES & STANDARDS

Agencies Affecting CNA Practice

Centers for Medicare & Medicaid Services (CMS)

Sets standards for nursing homes.

State Nurse Aide Registry

Tracks CNA certification.

State Department of Health

Monitors healthcare compliance.

Facility Policies

Every CNA must follow facility procedures.

Survey & Inspection Readiness

Facilities are monitored for:

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- Resident safety
- Abuse prevention
- Infection control
- Documentation

CNAs play an important role in compliance.

LESSON 8: ETHICAL DECISION-MAKING

When Facing Difficult Situations

Ask:

1. Is this safe?
 2. Is this respectful?
 3. Does this protect the resident?
 4. Should I report this?
-

Example Scenario

You observe rough handling of a resident.

Correct response:

- ☒ Protect resident and report concern.
-

LESSON 9: PROFESSIONAL ACCOUNTABILITY

Accountability Means:

Taking responsibility for actions.

CNAs should:

- ☒ Admit mistakes honestly
 - ☒ Report concerns promptly
 - ☒ Ask for help when unsure
-

Important Rule

When unsure—ask questions.

Resident safety comes first.

FINAL PROGRAM REFLECTION ACTIVITY

Reflection Prompt

You have completed the CNA course.

Reflect on:

1. What have you learned about compassionate caregiving?
 2. What skills do you feel most confident in?
 3. What areas do you want to continue improving?
-